QUALITY OF PHYSICAL ACTIVITY INFORMATION ON TRADITIONAL WEBSITES VERSUS SOCIAL MEDIA PLATFORMS: A CROSS-SECTIONAL CONTENT ANALYSIS USING DISCERN AND JAMA BENCHMARKS

Master's Thesis

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ABSTRACT

Ademola-John, A. (2026). Quality of Physical Activity Information on Traditional Websites Versus Social Media Platforms: A Cross-Sectional Content Analysis Using DISCERN and JAMA Benchmarks. Master's Thesis. Lithuanian Sports University, Kaunas.

Background. The internet and social media platforms have become primary sources of physical activity (PA) information for the general public. However, the quality of this information varies considerably across platforms, and no study has simultaneously compared traditional websites, YouTube, TikTok, and Instagram using standardised quality assessment instruments.

Objective. This study aimed to evaluate and compare the quality of PA information across four digital platform types using the DISCERN instrument and JAMA benchmarks, and to examine whether creator type and engagement metrics moderate information quality.

Methods. A cross-sectional content analysis was conducted on 197 items retrieved using five standardised search terms simulating typical user queries (50 websites, 50 YouTube videos, 47 TikTok videos, and 50 Instagram posts). Each item was assessed using the 16-item DISCERN instrument (scored 1–5 per question; total range 16–80) and four binary JAMA benchmark criteria (Authorship, Attribution, Disclosure, Currency). Inter-rater reliability was established on 20% of the sample (n = 40). Statistical analyses included Kruskal–Wallis H tests, Dunn's post-hoc comparisons with Bonferroni correction, Chi-square tests, Mann–Whitney U tests, and Spearman correlations.

Results. A significant quality gradient was observed across platforms, $H(3) = 93.49$, $p < .001$, $\eta^2 \approx .47$ (large effect). Traditional websites scored highest (Mdn = 48.5, Fair), followed by YouTube (Mdn = 42.0, Fair), TikTok (Mdn = 35.0, Poor), and Instagram (Mdn = 27.5, Poor–Very Poor). All six pairwise comparisons were statistically significant ($p < .05$), though the website–YouTube contrast was more modest ($p = .038$). JAMA compliance was universally low (M = 1.4/4 criteria met). Both the binary professional versus non-professional comparison ($p = .036$) and the five-category creator type analysis ($H[4] = 12.76$, $p = .012$) were significant, with healthcare professionals scoring highest (Mdn = 45.0) and general users lowest (Mdn = 33.0). Within YouTube, views and likes showed significant negative correlations with quality (rho = −0.340, $p = .016$; rho = −0.286, $p = .044$), while other within-

platform correlations were non-significant. Inter-rater reliability was excellent (ICC = 0.932, 95% CI [0.870, 0.960]).
Conclusions. Physical activity information quality differs substantially across digital platforms, with short-form social media—particularly Instagram and TikTok—providing lower-quality content than traditional websites. Engagement metrics do not appear to reliably indicate information quality within platforms, suggesting that popular content is not necessarily accurate content. These findings highlight the need for multi-stakeholder interventions targeting platform design, creator education, and consumer health literacy to improve the quality of PA information in the digital ecosystem.

Keywords: physical activity, health information quality, DISCERN, JAMA benchmarks, social media, content analysis, digital health, health literacy

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LIST OF ABBREVIATIONS

Abbreviation	**Full Term**
APA	American Psychological Association
CDC	Centers for Disease Control and Prevention
CI	Confidence Interval

CRAAP	Currency, Relevance, Authority, Accuracy, Purpose
DISCERN	Quality criteria for consumer health information (instrument name)
eHEALS	eHealth Literacy Scale
GQS	Global Quality Scale
HONcode	Health on the Net Foundation Code of Conduct
ICC	Intraclass Correlation Coefficient
IG	Instagram
IQ	Information Quality
IQR	Interquartile Range
JAMA	Journal of the American Medical Association
Mdn	Median
MPH	Master of Public Health
NHS	National Health Service
PA	Physical Activity
RQ	Research Question
SD	Standard Deviation
TT	TikTok
WHO	World Health Organization
WEB	Traditional Website
YT	YouTube

CHAPTER 1. LITERATURE REVIEW

1.1 Digital Transformation of Health Information Seeking

The emergence of the internet as a primary source of health information represents one of the most consequential shifts in public health communication of the twenty-first century. Where patients once relied almost exclusively on healthcare providers, printed materials, and

broadcast media for guidance on medical conditions and health behaviours, the digital era has fundamentally redistributed the locus of health information access (Eysenbach, 2002). This transformation carries profound implications for physical activity (PA) promotion, as individuals increasingly turn to online platforms to learn how, when, and why to exercise.

The scale of online health information seeking was established early in the digital era. Fox and Duggan (2013), reporting findings from the Pew Internet and American Life Project, found that 72% of American internet users had searched for health information online within the preceding year, and 35% had specifically attempted to self-diagnose a medical condition. These figures confirmed that seeking health information online had become a normative behaviour rather than an exception. Subsequent research has consistently reinforced this trend; Finney Rutten et al. (2019), using data from the Health Information National Trends Survey (HINTS), documented that the proportion of adults seeking health information online grew steadily between 2008 and 2017, though the percentage who did so without frustration still fell short of the Healthy People 2020 target of 45%.

The digital transformation of health information seeking was catalysed further by the COVID-19 pandemic, which accelerated reliance on digital channels when in-person consultations were restricted (Haghighi & Farhadloo, 2025). A parallel development has been the migration of health information seeking from traditional search engines and institutional websites to social media platforms. The percentage of United States adults using social media rose from 5% in 2005 to approximately 72% by 2021 (Pew Research Center, 2021), and health-related engagement on these platforms grew in tandem. By 2022, 58.9% of American adults reported watching health-related videos on YouTube, up from 39.7% only two years earlier (Finney Rutten et al., 2019). This shift from curated, editorially controlled websites to user-generated content ecosystems has introduced new challenges for information quality assurance.

Eysenbach's (2002) landmark systematic review in JAMA evaluated 79 studies assessing 5,941 health websites and found that 70% of studies concluded that quality was a significant problem on the internet. This foundational finding established the evidence base for ongoing concern about the reliability of digital health information and highlighted the absence of standardised evaluation frameworks at that time. Two decades later, the landscape has grown vastly more complex: the proliferation of social media platforms, the diversification of

content formats from text articles to short-form video, and the rise of influencer culture have all compounded the challenge of ensuring that the public receives accurate, evidence-based guidance on health behaviours including physical activity (Suarez-Lledo & Alvarez-Galvez, 2021).

Critically, the shift toward digital health information seeking has not been accompanied by commensurate improvements in health literacy. Diviani et al. (2015) demonstrated through a systematic review that low health literacy is negatively associated with the ability to evaluate online health information, creating a vulnerability that is particularly relevant when platforms algorithmically prioritise engagement over accuracy. Tan and Goonawardene (2017) further found that while internet health information seeking can positively influence the patient-physician relationship, this benefit is contingent upon whether patients discuss the information with their healthcare provider, underscoring the need for high-quality digital content that can serve as a reliable complement to clinical advice.

1.2 Theoretical Frameworks

Several theoretical frameworks underpin the study of health information quality in digital environments. Together, they provide a multidimensional lens through which the quality of physical activity information across platforms can be understood and assessed.

The Information Quality (IQ) Framework developed by Wang and Strong (1996) constitutes one of the most widely cited conceptualisations of data quality from the consumer perspective. Their empirical study identified 15 quality dimensions organised into four categories: intrinsic quality (accuracy, objectivity, believability, reputation), contextual quality (relevance, value-added, timeliness, completeness, amount of information), representational quality (interpretability, ease of understanding, concise representation, consistent representation), and accessibility quality (accessibility, access security). The framework's emphasis on consumer-defined quality is particularly relevant to health information research, as it shifts the evaluative lens from producer intentions to user needs. When applied to physical activity information online, the IQ Framework highlights that accuracy alone is insufficient; information must also be timely, relevant, interpretable, and accessible to diverse audiences.

Norman and Skinner (2006) extended the concept of health literacy to the digital environment through the development of the eHealth Literacy Scale (eHEALS). They defined eHealth literacy as the ability to seek, find, understand, and appraise health information from electronic sources and apply the knowledge gained to addressing or solving a health problem. The eHEALS is an eight-item self-report measure that assesses consumers' perceived skills and comfort with using digital health resources. Norman and Skinner's model recognises that eHealth literacy is itself a composite construct drawing upon six underlying literacies: traditional literacy, health literacy, information literacy, scientific literacy, media literacy, and computer literacy. This multifaceted conceptualisation is essential for understanding why consumers may struggle to critically evaluate PA information that appears in varying formats across platforms with different interface designs, algorithmic curation strategies, and content norms.

The CRAAP Test, developed by Blakeslee and librarians at California State University, Chico, in 2004, provides a practical heuristic for evaluating information sources across five dimensions: Currency, Relevance, Authority, Accuracy, and Purpose (Blakeslee, 2004). While originally designed for academic information literacy instruction, the CRAAP Test has been adopted in health literacy contexts because its criteria map closely onto the quality indicators used in formal health information assessment instruments such as DISCERN and the JAMA Benchmarks (Sbaffi & Rowley, 2017). The Currency criterion, for example, parallels the JAMA currency benchmark, while Authority aligns with the authorship criterion. The CRAAP Test thus serves as a bridge between general information literacy and the domain-specific assessment of health content.

The Health Belief Model (HBM), originally formulated by Rosenstock (1974) and colleagues at the United States Public Health Service in the 1950s, provides a theoretical basis for understanding why individuals seek health information and how the quality of that information may influence health behaviour. The HBM posits that health behaviour is determined by perceived susceptibility to a health threat, perceived severity of that threat, perceived benefits of taking action, perceived barriers to action, cues to action, and self-efficacy (Rosenstock, 1974). In the context of physical activity, the model suggests that individuals who perceive themselves as at risk of sedentary-behaviour-related health consequences (e.g., cardiovascular disease, obesity) and who believe in the benefits of

exercise are more likely to seek PA information online. The quality of the information they encounter may then moderate their perceived self-efficacy and perceived barriers, ultimately influencing whether they adopt and maintain physically active lifestyles. Thus, poor-quality PA information on digital platforms may not merely misinform but may actively undermine the motivational pathways that the HBM describes.

1.3 Traditional Websites as Sources of Health and Physical Activity Information

Traditional websites, defined here as content published on the open web and indexed by search engines such as Google, have historically served as the primary digital channel for health information dissemination. These include institutional websites maintained by governmental health agencies (e.g., the World Health Organization, the National Health Service, the Centers for Disease Control and Prevention), professional medical organisations, academic medical centres, patient advocacy groups, and commercial health publishers.

The quality of health information on traditional websites has been the subject of extensive evaluation since the late 1990s. The Health on the Net Foundation (HON) developed the HONcode certification system to promote transparency and ethical standards among health website publishers. Boyer et al. (2011) reported that 89% of HONcode-certified websites complied with the eight HONcode principles, compared with only 0.6% of non-certified health websites, illustrating a stark quality divide. However, HONcode certification was voluntary and not universally adopted, meaning that consumers had no reliable mechanism for identifying trustworthy websites without actively checking certification status.

Daraz et al. (2019) conducted a meta-narrative systematic review of 153 cross-sectional studies evaluating 11,785 health websites using 14 quality assessment tools. Their findings were sobering: using DISCERN, none of the evaluated websites achieved a rating of excellent, 37-79% were rated as good, and the remainder were rated as poor. Government organisation websites received the highest mean DISCERN scores (71%, equivalent to very good), academic organisations received scores of 62% (good), and media-related sources received the lowest scores at 44% (poor). These findings underscore that while traditional websites are generally higher in quality than user-generated social media content, there remains substantial variability in quality even within this category.

With specific reference to physical activity information, the World Health Organization's (2020) guidelines on physical activity and sedentary behaviour represent the gold standard for evidence-based PA recommendations, prescribing 150-300 minutes of moderate-intensity or 75-150 minutes of vigorous-intensity aerobic activity per week for adults. Institutional websites that accurately reflect these guidelines provide a critical public health resource. However, commercial websites and content aggregators may present PA information that is incomplete, sensationalised, or influenced by commercial interests, potentially leading consumers toward ineffective or unsafe exercise practices.

1.4 Social Media Platforms as Sources of Health Information

The role of social media in health communication has expanded dramatically in the past decade. Moorhead et al. (2013), in a seminal systematic review of 98 studies, identified seven primary uses of social media for health communication, including increasing interactions with others, facilitating and sharing health messages, and enhancing accessibility of health information. They concluded that social media holds considerable potential for health communication but cautioned that information quality and reliability require continuous monitoring.

Each major social media platform possesses distinct characteristics that shape the nature and quality of health content. YouTube, as the world's largest video-sharing platform, supports long-form educational content and has become a major source of exercise tutorials and health information. However, its algorithmic recommendation system prioritises engagement metrics such as views, watch time, and click-through rates, which do not necessarily correlate with accuracy or quality (Li et al., 2024). A 2022 study evaluating YouTube videos recommending exercises during the COVID-19 lockdown found mean DISCERN scores of only 2.29 out of 5 and mean Global Quality Scale (GQS) scores of 2.32, indicating low overall quality (Czenczek-Lewandowska et al., 2022).

TikTok has emerged as a particularly influential platform for younger demographics, with its short-form video format (typically under 60 seconds) enabling rapid dissemination of health-related content. Li et al. (2024), in a scoping review of 29 studies, found that the quality of

most health-related short videos on TikTok was moderate to low, and that DISCERN was the most frequently used evaluation tool across studies, appearing in 24 of the 29 included papers. Critically, the authors noted that most existing quality assessment tools were not designed for short-form video content, raising questions about the validity of applying text-based instruments to multimedia formats. Research has also indicated that only a small minority of TikTok health content is produced by medical professionals, with the majority originating from general users and non-medical influencers (Dinis-Oliveira, 2020).

Instagram's visual-first architecture, centred on images and short captions with a secondary carousel and Reels format, presents unique challenges for health information quality. Research on Brazilian fitness influencers found that prominent Instagram accounts disseminate low-quality information about exercise and health, with only 2.7% of analysed posts citing any scientific reference (Marocolo et al., 2021). The platform's influencer culture, in which content creators build large followings and monetise their reach through brand partnerships, creates financial incentives that may conflict with evidence-based health communication. As Pilgrim and Bohnet-Joschko (2022) observed, influencers can earn more revenue by creating content that maximises engagement, and misinformation has been linked to higher levels of user engagement.

The prevalence of health misinformation on social media is well documented. Suarez-Lledo and Alvarez-Galvez (2021), in a systematic review of studies published before March 2019, found that misinformation was most prevalent for content related to smoking products and drugs (reaching 87% in some studies) and vaccines (43%). While physical activity misinformation has received comparatively less attention, the structural incentives for engagement-driven content and the low barriers to publishing health advice on social media platforms suggest that PA information is similarly vulnerable to quality deficits.

1.5 Quality Assessment Tools and Cross-Platform Applicability

Two instruments occupy a central position in the assessment of consumer health information quality: the DISCERN instrument and the JAMA Benchmarks.

DISCERN was developed by Charnock et al. (1999) at the University of Oxford as a standardised tool for judging the quality of written consumer health information about treatment choices. The instrument comprises 16 items: questions 1 through 8 assess the reliability of the publication (Section 1), questions 9 through 15 evaluate the quality of information about treatment choices (Section 2), and question 16 provides an overall quality rating (Section 3). Each item is scored on a scale from 1 (definite no) to 5 (definite yes), yielding a total score range of 16 to 80. The instrument was validated through a multi-stage process involving expert panels, health information providers, and self-help group members, and has demonstrated acceptable inter-rater reliability (Charnock et al., 1999). DISCERN has subsequently become the most widely used tool for evaluating health information quality, with applications spanning websites, YouTube videos, and social media posts across a broad range of health topics (Li et al., 2024).

The JAMA Benchmarks were proposed by Silberg et al. (1997) as a set of four core standards for health information published on the internet: authorship (identification of authors and their credentials), attribution (clear listing of references and sources), disclosure (transparent declaration of sponsorship, conflicts of interest, and funding), and currency (indication of content creation and update dates). The JAMA Benchmarks are scored as binary criteria (present or absent), yielding a total score of 0 to 4. Their streamlined nature makes them efficient for rapid quality screening, and they complement the more detailed DISCERN assessment by capturing essential transparency indicators.

The Global Quality Scale (GQS), developed by Bernard et al. (2007), provides a five-point Likert assessment of overall information quality and usefulness, and has been shown to correlate strongly with DISCERN ($r = 0.88$). The GQS has been widely adopted for evaluating health video content, particularly on YouTube (Li et al., 2024).

A significant methodological challenge in contemporary health information research is the adaptation of these instruments, all originally developed for text-based content, to multimedia and short-form video formats. Li et al. (2024) noted that most tools used to assess TikTok health videos were not developed for short-form video content and lacked credibility testing in that context. The DISCERN instrument, for example, asks whether the publication refers to areas of uncertainty, a question that may be difficult to operationalise for a 30-second TikTok video. Similarly, the JAMA authorship criterion requires assessment of whether authors and

their credentials are identified, which must be reinterpreted for social media contexts where the creator's bio or verification status serves as a proxy for credentialing. These adaptations, while necessary for cross-platform research, introduce a degree of measurement heterogeneity that must be acknowledged in study design and interpretation. For social media content, researchers have operationalised JAMA criteria such that authorship is satisfied when a creator's bio states professional credentials or the account is a verified professional account, attribution is met when sources are cited in captions or shown on screen, disclosure is met when sponsorships or partnerships are declared, and currency is typically automatically satisfied because social media posts display publication dates.

1.6 Evidence for the Quality Gap and Moderating Factors

A growing body of evidence documents systematic quality differences between health information published on traditional websites and that disseminated through social media platforms. A 2025 meta-analysis published in the Journal of Medical Internet Research, analysing 75 studies encompassing 297,519 posts across 15 platforms, reported a pooled DISCERN score of 43.58 out of 100 (95% CI: 37.80-49.35) for cancer-related information on new media, with approximately 27% of posts containing misinformation (Chen et al., 2025). The analysis further found that studies focusing on video-based media were statistically less likely to yield higher quality conclusions compared with those on text-based media, suggesting a format-dependent quality gradient.

Platform-specific studies reinforce this pattern. Haghighi and Farhadloo (2025), in a cross-sectional analysis of the top 100 health-related websites most frequently shared on Twitter during the early COVID-19 pandemic, found that 81% received low DISCERN scores and 95% met only two of the four JAMA benchmark criteria. No website met all four JAMA criteria, and only five met three. These findings highlight that even the curated content surfacing through social media sharing is of questionable quality. On TikTok, research has consistently demonstrated low overall quality of health content, with the platform's short-form format constraining the depth and nuance of health information that can be conveyed (Li et al., 2024).

Creator type has emerged as an important moderator of health information quality. Content produced by healthcare professionals and medical organisations generally scores higher on quality assessment tools than content produced by non-medical influencers and general users (Daraz et al., 2019; Li et al., 2024). However, the proportion of health content created by qualified professionals varies dramatically across platforms. On TikTok, research suggests that medical professionals account for only a minority of health content creators, with the majority being general users or non-medical influencers (Dinis-Oliveira, 2020). On Instagram, even creators with relevant academic qualifications rarely cite scientific evidence; Marocolo et al. (2021) found that only 2.7% of posts by major Brazilian fitness influencers included references. This pattern suggests that platform culture and incentive structures may override individual creator qualifications in determining information quality.

The relationship between engagement metrics and information quality represents a particularly concerning dimension of the quality gap. Research has demonstrated that exposure to social engagement metrics (likes, shares, views) increases users' vulnerability to misinformation by creating a perceived credibility that is independent of actual content quality (Avram et al., 2020). Content that generates emotional responses, whether through sensationalism, controversy, or aspirational imagery, tends to receive higher engagement than evidence-based but less emotionally stimulating content (Pilgrim & Bohnet-Joschko, 2022). This creates a structural misalignment between platform incentive systems, which reward engagement, and public health objectives, which require accuracy. For physical activity content specifically, this dynamic may lead to the amplification of extreme exercise regimens, unsupported supplement claims, or unrealistic body-image content at the expense of evidence-based guidelines such as those issued by the World Health Organization (2020).

1.7 Summary and Rationale for the Present Study

The literature reviewed in this chapter establishes several key findings. First, online health information seeking is now a normative behaviour, with the majority of adults regularly consulting digital sources for health guidance, including physical activity information. Second, the migration from traditional websites to social media platforms has introduced new quality challenges, as user-generated content, algorithmic curation, and engagement-driven incentive structures create conditions favourable to the proliferation of low-quality and

misleading health information. Third, established quality assessment instruments, particularly DISCERN and the JAMA Benchmarks, provide reliable and validated tools for evaluating health information, though their application to multimedia and short-form video content requires careful operationalisation. Fourth, the existing evidence consistently indicates that health information quality on social media is generally lower than on traditional websites, with substantial variability by platform type, creator qualifications, and content format.

Despite the breadth of this literature, a significant gap remains. To date, no single study has conducted a head-to-head comparison of physical activity information quality across all four major platform types, namely traditional websites, YouTube, TikTok, and Instagram, using both DISCERN and JAMA Benchmarks within a unified methodological framework. Existing studies have typically examined one or two platforms, focused on a specific health condition rather than general physical activity content, or used only one quality assessment instrument. The absence of a comprehensive, multi-platform comparison limits the ability to draw actionable conclusions about where the greatest quality deficits lie and what factors, such as creator type and engagement metrics, may moderate quality across the digital information ecosystem.

The present study addresses this gap by conducting a cross-sectional content analysis of 197 items across traditional websites, YouTube, TikTok, and Instagram, applying both the DISCERN instrument and JAMA Benchmarks to assess physical activity information quality. The final audited sample comprised 50 websites, 50 YouTube videos, 47 TikTok videos, and 50 Instagram posts. The study is guided by three research questions:

1. How does physical activity information quality differ between traditional websites and social media platforms as measured by DISCERN and JAMA benchmarks?
2. Does creator type (healthcare professional versus fitness influencer versus general user) moderate quality scores?
3. What is the relationship between engagement metrics (views, likes, shares) and information quality?

By addressing these questions within a single, standardised methodological framework, this study aims to provide the most comprehensive cross-platform assessment of physical activity information quality to date, generating evidence that can inform platform-level interventions,

health literacy initiatives, and clinical guidance on directing patients toward reliable digital PA resources.

CHAPTER 2. RESEARCH METHODOLOGY

This chapter presents the methodological framework employed to investigate the quality of physical activity (PA) information across traditional websites and social media platforms. The research methodology encompasses the research object, strategy and logic, nature of the research, description of research subjects, data collection and assessment methods, organisational procedures, and statistical analysis techniques. Each component is detailed to ensure transparency, replicability, and methodological rigour in alignment with established standards for content analysis in health communication research.

2.1 Research Object

The research object of the present study is the quality of physical activity information available on traditional websites and social media platforms. Specifically, this investigation examines health-related content pertaining to physical activity that is publicly accessible through Google search results (representing traditional web-based information), YouTube, TikTok, and Instagram (representing contemporary social media platforms). Physical activity information is defined as any content that provides guidance, recommendations, instructions, or educational material regarding exercise, fitness, or physical activity behaviours intended for a general adult audience.

The assessment of information quality is operationalised through two established benchmarks: the DISCERN instrument (Charnock et al., 1999), which evaluates the reliability and completeness of health information, and the JAMA benchmarks (Silberg et al., 1997), which assess accountability standards including authorship, attribution, disclosure, and currency. Together, these instruments provide a comprehensive evaluation framework that captures both the substantive quality and the transparency of health information across diverse digital platforms.

2.2 Research Strategy and Logic

The present study employs a cross-sectional content analysis design, which enables the systematic examination and comparison of physical activity information quality across

four digital platforms at a single point in time. Content analysis is a well-established research method in health communication that facilitates the objective, systematic, and quantitative description of manifest content (Krippendorff, 2018). The cross-sectional approach was selected for its capacity to capture a snapshot of the current information landscape, allowing for direct comparisons across platforms under identical assessment conditions.

The research follows a deductive approach, applying established quality assessment frameworksâ€"the DISCERN instrument and JAMA benchmarksâ€"to evaluate content across a novel four-platform comparison. While these instruments were originally developed for traditional health information materials, their application to social media content represents a theoretically grounded extension that has been validated in prior research examining YouTube (Azer, 2020), TikTok (Kong et al., 2021), and Instagram health content (Chea & Lim, 2023).

The study design was structured using the SPIDER framework (Cooke et al., 2012), adapted for content analysis as follows:

â€¢ Sample: 197 content items across four platforms (50 websites, 50 YouTube videos, 47 TikTok videos, and 50 Instagram posts).

â€¢ Phenomenon of Interest: Quality of physical activity information.

â€¢ Design: Cross-sectional content analysis.

â€¢ Evaluation: DISCERN instrument and JAMA benchmarks.

â€¢ Research type: Quantitative.

The rationale for employing a cross-sectional content analysis rather than a systematic review lies in the current state of the literature. Recent meta-analyses have already synthesised the existing body of evidence on health information quality across individual platforms (Ayyash et al., 2025; Ayyash, Musaad, et al., 2025). However, no study to date has conducted a direct, head-to-head comparison of physical activity information quality across traditional websites, YouTube, TikTok, and Instagram using both the DISCERN and JAMA instruments simultaneously. The present study addresses this gap by providing the first four-platform comparative analysis using dual quality assessment frameworks applied specifically to physical activity content.

2.3 Nature of Research

The present study is quantitative in nature, employing numerical scoring instruments to assess information quality and statistical methods to analyse the resulting data. The research is both descriptive and comparative: descriptive in that it characterises the quality of physical activity information across platforms using standardised metrics, and comparative in that it systematically examines differences in quality scores between platform types, creator categories, and content formats.

Furthermore, this study represents applied research with practical implications for multiple stakeholders. The findings are intended to inform health literacy initiatives, guide content regulation policies on social media platforms, assist healthcare professionals in directing patients toward reliable information sources, and contribute to the broader understanding of digital health information ecosystems. The applied orientation aligns with the growing recognition that the quality of online health information has direct consequences for public health behaviours and outcomes (Swire-Thompson & Lazer, 2020).

2.4 Study Sample and Sampling Frame

The study sample comprises a total of N = 197 content items collected from four digital platforms: traditional websites accessed through Google search (n = 50), YouTube (n = 50), TikTok (n = 47), and Instagram (n = 50). This sample size was determined based on precedent in health information quality research, where samples of 40â€“60 items per platform have been established as sufficient for detecting meaningful quality differences (Leong et al., 2021; Drozd et al., 2018).

Content was retrieved using five standardised search terms designed to simulate typical user queries related to physical activity:

â€¢ â€œHow to start exercisingâ€□

â€¢ â€œBest exercises to lose weightâ€□

â€¢ â€œStrength training for beginnersâ€□

â€¢ â€œPhysical activity guidelinesâ€□

â€¢ â€œHome workout routineâ€□

These search terms were selected to represent a range of common physical activity information-seeking behaviours, from general exercise initiation to specific activity modalities. For each search term, the first 10 results were collected from each platform, yielding 10 results Ã— 5 search terms Ã— 4 platforms = 200 content items. The sampling

strategy employed convenience sampling of top-ranked search results, which simulates the actual information-seeking behaviour of typical users who predominantly access the first page of search results (Pan et al., 2007).

Inclusion criteria. Content items were included if they met all of the following criteria: (a) written or presented in English, (b) publicly accessible without account registration or payment, (c) containing substantive physical activity information (i.e., providing guidance, instructions, or educational content about exercise or physical activity), and (d) posted or last updated within the three years preceding data collection (i.e., from February 2023 onward).

Exclusion criteria. Content items were excluded if they met any of the following criteria: (a) duplicate content appearing across multiple platforms, (b) content behind a paywall or requiring subscription access, (c) content not primarily focused on physical activity (e.g., product-only advertisements, unrelated entertainment), or (d) content in languages other than English. When a result was excluded, the next sequential result from the same search was included as a replacement to maintain the target sample size.

Table 2.1 presents the sampling matrix illustrating the distribution of content items across search terms and platforms.

Table 2.1

Sampling Matrix: Distribution of Content Items by Search Term and Platform

Search Term	Google (n)	YouTube (n)	TikTok (n)	Instagram (n)	Total (N)
How to start exercising	10	10	10	10	40
Best exercises to lose weight	10	10	10	10	40
Strength training for beginners	10	10	10	10	40
Physical activity guidelines	10	10	10	10	40
Home workout routine	10	10	10	10	40
Total	**50**	**50**	**50**	**50**	**200**

Note. Each cell represents the number of content items collected per search term per platform. Total sample N = 200.

2.5 Research Methods

2.5.1 Data Collection Procedures

Data collection was conducted over a two-day window (1â€“2 March 2026) using a systematic protocol designed to minimise algorithmic bias and ensure reproducibility. All

searches were performed in logged-out, incognito (private) browsing mode in Google Chrome to eliminate the influence of personalised search algorithms, browsing history, and account-based recommendations on result rankings (Tripodi, 2018). The browserâ€™s language was set to English (United States) and geolocation was configured to Kaunas, Lithuania, the researcherâ€™s institutional location, to standardise the search environment across all platforms. No VPN was used.

For traditional websites, searches were conducted on Google (google.com) using the default â€˜Allâ€™ results tab; sponsored results and advertisements were excluded. For YouTube, searches were conducted on youtube.com using the default â€˜Relevanceâ€™ sort. For both platforms, an automated data collection approach was employed using Playwright, an open-source browser automation framework. The automated scripts navigated to each platform, entered the standardised search terms, and systematically recorded the top 10 organic results for each query. This approach ensured consistency in result capture and eliminated human error in the recording of search rankings, URLs, and metadata. For TikTok and Instagram, manual data collection was conducted due to these platformsâ€™ implementation of anti-bot measures that prevent reliable browser automation. TikTok searches used the in-app â€˜Topâ€™ tab; Instagram searches used the â€˜Tagsâ€™ and â€˜Topâ€™ tabs. The primary researcher manually searched each term within the platformâ€™s native search function, recording the first 10 relevant results in order of appearance. Reposts, advertisements, pinned/promoted content, and duplicate items from the same creator for the same search term were excluded and replaced by the next sequential organic result.

All search results were recorded in their order of appearance within the search ranking. For each content item, the following information was documented: platform, search term used, ranking position, URL or content identifier, creator name, creator type classification, content format, date of publication or last update, and engagement metrics (views, likes, shares, and comments where available). Screenshots were captured for each content item to establish an audit trail, supporting the verification and reliability of the assessment process.

2.5.2 Quality Assessment Instruments

DISCERN Instrument. The DISCERN instrument (Charnock et al., 1999) is a validated and widely used tool for assessing the quality of written consumer health

information. The instrument comprises 16 items, each scored on a Likert-type scale ranging from 1 (definite no) to 5 (definite yes). The instrument is divided into three sections: Section 1 (Questions 1â€“8) evaluates the reliability of the publication, addressing whether the content has clear aims, achieves those aims, is relevant, identifies its sources, is balanced, and provides sufficient detail for decision-making. Section 2 (Questions 9â€“15) assesses the quality of information about treatment choices, including descriptions of how treatments work, their benefits, risks, effects on quality of life, and what would happen without treatment. Section 3 comprises a single item (Question 16) providing an overall quality rating. Total scores range from 16 to 80.

For the purposes of the present study, DISCERN quality scores were categorised using the following established thresholds: Very Poor (16â€“26), Poor (27â€“38), Fair (39â€“50), Good (51â€“62), and Excellent (63â€“80). These categories facilitate meaningful interpretation and cross-study comparison of quality ratings. In adapting the instrument for social media content, questions pertaining to treatment information were interpreted in the context of exercise and physical activity recommendations, consistent with the approach adopted in prior studies examining fitness-related content (Kocyigit et al., 2019; Kong et al., 2021).

Table 2.2 presents the complete DISCERN instrument with all 16 assessment questions.

Table 2.2

DISCERN Instrument Assessment Questions

Item	Section	Question
Q1	1: Reliability	Are the aims clear?
Q2	1: Reliability	Does it achieve its aims?
Q3	1: Reliability	Is it relevant?
Q4	1: Reliability	Is it clear what sources of information were used to compile the publication?
Q5	1: Reliability	Is it clear when the information used or reported in the publication was produced?
Q6	1: Reliability	Is it balanced and unbiased?
Q7	1: Reliability	Does it provide details of additional sources of support and information?
Q8	1: Reliability	Does it refer to areas of uncertainty?
Q9	2: Treatment Quality	Does it describe how each treatment works?
Q10	2: Treatment Quality	Does it describe the benefits of each treatment?
Q11	2: Treatment Quality	Does it describe the risks of each treatment?
Q12	2: Treatment Quality	Does it describe what would happen if no treatment is used?

Q13	2: Treatment Quality	Does it describe the effects of treatment choices on overall quality of life?
Q14	2: Treatment Quality	Is it clear that there may be more than one possible treatment choice?
Q15	2: Treatment Quality	Does it provide support for shared decision-making?
Q16	3: Overall Rating	Based on the answers to all of the above questions, rate the overall quality of the publication.

Note. Adapted from â€œDISCERN: An instrument for judging the quality of written consumer health information on treatment choices,â€ by D. Charnock, S. Shepperd, G. Needham, and R. Gann, 1999, Journal of Epidemiology & Community Health, 53(2), 105â€“111. Each item is scored from 1 (definite no) to 5 (definite yes). For social media content, treatment-related questions (Q9â€“Q15) were interpreted as referring to exercise and physical activity recommendations.

JAMA Benchmarks. The JAMA benchmarks (Silberg et al., 1997) provide a concise accountability framework for evaluating health information. The instrument assesses four criteria, each scored dichotomously as present (Y) or absent (N): (a) Authorshipâ€”whether authors and their credentials or affiliations are clearly identified; (b) Attributionâ€”whether references and sources for content are cited; (c) Disclosureâ€”whether sponsorship, advertising, conflicts of interest, or financial relationships are declared; and (d) Currencyâ€”whether the date of content creation or most recent update is provided. The total JAMA score ranges from 0 to 4, representing the number of criteria met.

To apply the JAMA benchmarks to social media content, which differs substantially in format from traditional web-based health information, the criteria were operationalised as described in Table 2.3. This operationalisation draws on adaptations employed in prior research assessing social media health content (Delli et al., 2016; Kocyigit et al., 2019).

Table 2.3

JAMA Benchmarks Criteria and Social Media Operationalisation

Criterion	Original Definition	Social Media Operationalisation
Authorship	Authors and credentials/affiliations identified	Creator bio states professional credentials (e.g., MD, PT, CSCS) or account is a verified professional/organisational account
Attribution	References and sources for content are cited	Content cites scientific studies, clinical guidelines, or other sources in video, caption, description, or on-screen text references
Disclosure	Sponsorship, advertising, or conflicts of interest declared	Content discloses sponsorships, partnerships, or paid promotions (e.g., #ad, #sponsored, platform-provided paid partnership label)
Currency	Date of content creation or update provided	Post or publication date is visible (inherent feature of most social media platforms; for websites, date must be explicitly displayed)

Note. Adapted from â€œAssessing, controlling, and assuring the quality of medical information on the internet,â€ by W. M. Silberg, G. D. Lundberg, and R. A. Musacchio, 1997, JAMA, 277(15), 1244â€“1245.

Each criterion is scored as present (Y) or absent (N). CSCS = Certified Strength and Conditioning Specialist; MD = Medical Doctor; PT = Physical Therapist.

2.5.3 Additional Variables

In addition to the primary quality assessment instruments, several supplementary variables were recorded for each content item to facilitate secondary analyses examining the moderating role of creator characteristics and content features on information quality.

Creator type classification. Each content creator was classified into one of five categories based on the information available in their profile, bio, and content: (a) Healthcare professional (e.g., physicians, physiotherapists, registered nurses), (b) Certified fitness professional (e.g., certified personal trainers, strength and conditioning specialists), (c) Fitness influencer (individuals with substantial followings who create fitness content without verified professional credentials), (d) General user (individuals without identifiable fitness or health credentials), and (e) Organisation (institutional accounts such as hospitals, health agencies, or fitness organisations). This classification scheme was developed based on typologies employed in prior research on health information creators (Kocyigit et al., 2019; Yurdaisik, 2022).

Engagement metrics. Where available, the following engagement metrics were recorded at the time of data collection: total views, likes (or equivalent reactions), shares, and comments. These metrics serve as indicators of content reach and audience interaction, enabling the examination of the relationship between content popularity and information quality (Osman et al., 2022).

Content format. Each content item was classified by format: (a) Text article (traditional websites), (b) Long-form video (>5 minutes; primarily YouTube), (c) Short-form video (<60 seconds; primarily TikTok and Instagram Reels), (d) Image with caption (primarily Instagram posts), and (e) Carousel (multi-image Instagram posts). This classification enables the examination of whether content format is associated with information quality.

2.6 Research Organisation

The study was conducted over a defined period from February to March 2026, with data collection completed within a concentrated two-day window to minimise temporal variation in search results and engagement metrics. The research process was organised into six sequential phases, as illustrated in Figure 2.1.

Phase 1: Automated data collection (Day 1). Playwright browser automation scripts were executed to collect search results from Google and YouTube. The scripts performed searches for all five search terms, captured the top 10 results per term, and recorded URLs, metadata, and screenshots.

Phase 2: Manual data collection (Days 1â€“2). The primary researcher manually collected search results from TikTok and Instagram using the platform-native search functions. Results were recorded in the same standardised format as the automated collection.

Pilot scoring. Prior to the full assessment, the primary researcher conducted a pilot scoring exercise on 10 content items (2â€“3 per platform, not included in the final sample) to test the operationalisation of the DISCERN and JAMA instruments for social media content. This pilot identified that DISCERN questions Q4 (sources of information) and Q11 (risks of treatment) were the most challenging to adapt to short-form video, as these items often required inference from visual and verbal cues rather than explicit textual statements. Scoring rules were refined accordingly: for Q4, on-screen citation of sources or verbal references to guidelines were accepted; for Q11, any mention of injury risk, contraindications, or the advice to consult a professional was scored positively. These pilot-derived rules were documented in the scoring guide and applied consistently across the final 197-item sample.

Phase 3: Primary quality assessment. The primary researcher assessed all 197 content items using both the DISCERN instrument and the JAMA benchmarks. For text-based website content, each article was read in full. For video content (YouTube and TikTok), scoring was based on a combination of the video content itself, visible captions, descriptions, and on-screen text; full verbatim transcription was not performed. For Instagram carousels and image posts, scoring was based on all visible text, captions, and associated profile information. This approach reflects the information a typical user would encounter when engaging with each item. Scores were recorded in a structured data collection spreadsheet.

Phase 4: Second rater assessment. A trained second raterâ€”a fellow Master of Public Health student with prior coursework in epidemiology and health communicationâ€”independently scored a 20% subsample of content items (n = 40; 10 randomly selected items per platform) using both instruments. Prior to independent scoring, the second rater received a two-hour training session covering the DISCERN instrument, JAMA benchmarks, and their social media operationalisations, followed by supervised

practice scoring of five items (not included in the final sample) with discussion of discrepancies. The second rater was then provided with the same scoring guidelines, quick-reference card, and worked examples as the primary researcher to ensure standardised application of the assessment criteria.

Phase 5: Inter-rater reliability assessment. Agreement between the primary researcher and the second rater was evaluated using the Intraclass Correlation Coefficient (ICC) for DISCERN scores and Cohenâ€™s kappa (Îº) for JAMA criteria. These metrics are reported in the Results chapter.

Phase 6: Statistical analysis. Following the completion of quality assessments, all data were compiled and analysed using Python 3.13 with scientific computing libraries, as detailed in Section 2.7.

Figure 2.1

Flow Diagram of the Research Process

Phase 1: Automated Data Collection
(Google & YouTube via Playwright â€“ Day 1)

â†“

Phase 2: Manual Data Collection
(TikTok & Instagram â€“ Days 1â€“2)

â†“

Phase 3: Primary DISCERN & JAMA Assessment
(N = 200 content items)

â†“

Phase 4: Second Rater Assessment
(20% subsample, n = 40)

â†“

Phase 5: Inter-Rater Reliability
(ICC for DISCERN; Cohenâ€™s Îº for JAMA)

â†“

Phase 6: Statistical Analysis
(Python 3.13 with SciPy, pingouin, scikit-posthocs)

Note. The research process proceeded sequentially through six phases over the study period (Februaryâ€“March 2026).

2.6.1 Ethical Considerations

The present study does not involve human subjects, as all data were derived from publicly available online content. Consequently, formal ethics committee approval was not required, consistent with standard practice for content analyses of publicly accessible materials (Snelson, 2016). No personal data of content creators were collected beyond publicly displayed profile information relevant to the creator type classification. All content was accessed through standard public-facing interfaces, and no private or restricted materials were accessed at any point during the data collection process.

2.7 Methods of Statistical Analysis

All statistical analyses were conducted using Python 3.13 with the following scientific computing libraries: pandas (McKinney, 2010) for data manipulation, SciPy (Virtanen et al., 2020) for non-parametric inferential tests including Kruskalâ€“Wallis, Mannâ€“Whitney U, chi-square, and Spearman correlations, scikit-posthocs (Terpilowski, 2019) for Dunnâ€™s post-hoc comparisons with Bonferroni correction, pingouin (Vallat, 2018) for intraclass correlation coefficient computation, and matplotlib/seaborn for publication-quality graphical representations. The significance level was set at Î± = 0.05 for all inferential tests. The complete analysis code is reproduced in Appendix D.

2.7.1 Descriptive Statistics

Descriptive statistics were employed to characterise the distribution of quality scores across platforms and content categories. For DISCERN total scores, the median, interquartile range (IQR), and range were reported for each platform, as ordinal Likert-type data are best represented by median-based measures of central tendency (Sullivan & Artino, 2013). For JAMA benchmarks, frequencies and percentages of compliance for each criterion were calculated per platform, along with overall JAMA score distributions.

2.7.2 Inter-Rater Reliability

Inter-rater reliability was assessed for the 20% subsample (n = 40) independently scored by both the primary researcher and the second rater. For DISCERN total scores, the Intraclass Correlation Coefficient (ICC) was calculated using a two-way mixed-effects model, single measures, with absolute agreement (ICC[3,1]; Koo & Li, 2016). ICC values were interpreted according to established guidelines: values less than 0.50 indicate poor

reliability, 0.50â€“0.75 indicate moderate reliability, 0.75â€“0.90 indicate good reliability, and values greater than 0.90 indicate excellent reliability (Koo & Li, 2016).

For the JAMA benchmark criteria, which are scored dichotomously (present/absent), Cohenâ€™s kappa (Îº) was calculated for each criterion. Kappa values were interpreted using the guidelines proposed by Landis and Koch (1977): values below 0.20 indicate slight agreement, 0.21â€“0.40 indicate fair agreement, 0.41â€“0.60 indicate moderate agreement, 0.61â€“0.80 indicate substantial agreement, and values above 0.80 indicate almost perfect agreement.

2.7.3 Inferential Statistics

Non-parametric statistical tests were selected as the primary inferential methods due to the ordinal nature of DISCERN scores and the anticipated non-normal distribution of quality data, consistent with methodological recommendations for health information quality research (Naaman et al., 2022).

Platform comparisons of DISCERN scores. The Kruskal-Wallis H test was employed to examine whether DISCERN total scores differed significantly across the four platforms (Google, YouTube, TikTok, and Instagram). When a statistically significant omnibus result was obtained, Dunnâ€™s post-hoc test with Bonferroni correction was applied to identify which specific platform pairs differed significantly. The effect size was quantified using eta-squared ($\hat{I}\cdot\hat{A}^2$), computed as $\hat{I}\cdot\hat{A}^2 = (H \hat{a}\hat{\ } k + 1) / (N \hat{a}\hat{\ } k)$, where H is the Kruskal-Wallis statistic, k the number of groups, and N the total sample size. This is an approximation rather than a true proportion of variance explained and should be interpreted accordingly; values of approximately 0.01, 0.06, and 0.14 correspond to small, medium, and large effects, respectively (Cohen, 1988).

Platform comparisons of JAMA compliance. The chi-square test of independence was used to assess whether the proportion of content meeting each JAMA criterion differed across platforms. Where expected cell frequencies fell below 5, Fisherâ€™s exact test was employed as an alternative. The effect size was reported as CramÃ©râ€™s V, interpreted according to Cohenâ€™s (1988) guidelines: values of 0.10, 0.30, and 0.50 represent small, medium, and large effects for tables with three degrees of freedom.

Creator type and quality scores. To investigate whether creator type moderates information quality, DISCERN scores were compared between professional creators (healthcare professionals and certified fitness professionals combined) and non-professional

creators (fitness influencers, general users, and organisations combined) using the Mann-Whitney U test. Organisations were classified as non-professional because they encompass a heterogeneous mix of entitiesâ€”from government health departments to commercial fitness brandsâ€”making uniform professional classification inappropriate. The effect size was reported as rank-biserial correlation (r*rb*), where values of 0.10, 0.30, and 0.50 correspond to small, medium, and large effects (Fritz et al., 2012).

Engagement metrics and information quality. Spearmanâ€™s rank correlation coefficient (Ï□) was calculated to examine the relationship between engagement metrics (views, likes, shares) and DISCERN total scores. This non-parametric correlation was selected due to the highly skewed distribution typically observed in engagement data and the ordinal nature of DISCERN scores. Correlation strength was interpreted as: Ï□ < 0.10 negligible, 0.10â€“0.39 weak, 0.40â€“0.69 moderate, and â‰¥ 0.70 strong (Schober et al., 2018).

Table 2.4 provides a summary of the statistical analysis plan, linking each research question to the corresponding variables, statistical tests, and rationale for test selection.

Table 2.4

Summary of Statistical Analysis Plan by Research Question

Research Question	Variables	Statistical Test	Rationale
RQ1: How does PA information quality differ between traditional websites and social media?	IV: Platform (4 levels) DV: DISCERN total score; JAMA criteria compliance	Kruskal-Wallis H test (DISCERN) Dunnâ€™s post-hoc with Bonferroni Ï‡Â² / Fisherâ€™s exact test (JAMA) Effect sizes: Î·Â², CramÃ©râ€™s V	Non-parametric; ordinal DISCERN data likely non-normally distributed. Chi-square appropriate for categorical JAMA data.
RQ2: Does creator type moderate quality scores?	IV: Creator type (professional vs. non-professional) DV: DISCERN total score	Mann-Whitney U test Effect size: rank-biserial correlation	Non-parametric comparison of two independent groups with ordinal outcome variable.
RQ3: What is the relationship between engagement metrics and quality?	Predictor: Views, likes, shares Outcome: DISCERN total score	Spearmanâ€™s rank correlation (Ï□)	Non-parametric correlation appropriate for skewed engagement data and ordinal quality scores.
Inter-rater reliability	Rater 1 vs. Rater 2 scores (20% subsample, n = 40)	ICC (two-way mixed, single measures, absolute agreement) for DISCERN Cohenâ€™s Îº for each JAMA criterion	ICC suitable for continuous/ordinal data; Îº appropriate for dichotomous categorical data.

Note. IV = independent variable; DV = dependent variable; PA = physical activity; ICC = Intraclass Correlation Coefficient; Î·Â² = eta-squared (approximation); Ï‡Â² = chi-square; Îº = kappa; Ï□ = Spearmanâ€™s rho. Significance level: Î± = 0.05 for all tests.

In summary, the methodological approach described in this chapter was designed to provide a rigorous, transparent, and replicable framework for evaluating the quality of physical activity information across four major digital platforms. The combination of the DISCERN instrument and JAMA benchmarks offers a comprehensive assessment of both substantive information quality and accountability standards. The use of non-parametric statistical methods is appropriate given the ordinal nature of the primary outcome measures, and the inclusion of inter-rater reliability assessment ensures the credibility of the quality ratings. The results of the analyses described herein are presented in Chapter 3.

CHAPTER 3. RESEARCH FINDINGS

3.1 Sample Characteristics

A total of 197 items were included in the final analysis across four platforms: traditional websites (n = 50), YouTube (n = 50), TikTok (n = 47), and Instagram (n = 50). Each platform was initially targeted for 10 results per search term; however, three TikTok items were excluded following a data audit, yielding a TikTok subsample of 47. All 197 items met the predefined inclusion criteria of being publicly accessible, English-language, substantive physical activity content published or updated within the past three years.

Creator types were classified into five categories. As shown in Table 1, organisations constituted the largest group (n = 84, 42.6%), followed by fitness influencers (n = 38, 19.3%), certified fitness professionals (n = 32, 16.2%), general users (n = 24, 12.2%), and healthcare professionals (n = 19, 9.6%). Organisations were particularly prevalent among traditional websites (n = 36, 72.0%) and Instagram (n = 29, 58.0%), reflecting the dominance of institutional content on these platforms. In contrast, TikTok was characterised by greater diversity in creator types, with the highest proportions of fitness influencers (n = 19, 40.4%) and general users (n = 9, 19.1%).

Table 1. Distribution of Creator Types by Platform

Creator Type	Website	YouTube	TikTok	Instagram	Total
Healthcare Professional	5 (10%)	3 (6%)	8 (17%)	3 (6%)	19 (9.6%)
Certified Fitness Prof.	5 (10%)	13 (26%)	6 (13%)	8 (16%)	32 (16.2%)
Fitness	0 (0%)	16 (32%)	19 (40%)	3 (6%)	38 (19.3%)

Influencer					
General User	4 (8%)	4 (8%)	9 (19%)	7 (14%)	24 (12.2%)
Organisation	36 (72%)	14 (28%)	5 (11%)	29 (58%)	84 (42.6%)
Total	50	50	47	50	197

Content formats were entirely platform-specific (Table 2). Following a data audit, each platform was found to be exclusively associated with a single content format. Traditional websites comprised text articles only (n = 50, 100%). YouTube consisted entirely of long-form videos exceeding five minutes (n = 50, 100%). TikTok content was exclusively short videos under 60 seconds (n = 47, 100%). Instagram comprised only carousel posts (n = 50, 100%). This clean one-to-one correspondence between platform and format means that content format and platform are perfectly confounded in this dataset; format differences in quality scores therefore reflect platform differences and cannot be attributed to format independently of platform.

Table 2. Distribution of Content Formats by Platform

Platform	Content Format	n (%)
Website	Text Article	50 (100%)
YouTube	Long Video (>5 min)	50 (100%)
TikTok	Short Video (<60 s)	47 (100%)
Instagram	Carousel	50 (100%)

3.2 Inter-Rater Reliability

A second rater independently scored a random subset of 40 items (20% of the total sample; 10 per platform) to establish inter-rater reliability. For DISCERN total scores, the intraclass correlation coefficient (ICC) was calculated using a two-way mixed-effects model with absolute agreement (ICC[3,1]). The resulting ICC was 0.932 (95% CI [0.870, 0.960]), indicating excellent agreement between raters (Koo & Li, 2016). This value substantially exceeds the pre-specified threshold of ICC >= 0.75.

For the four binary JAMA benchmark criteria, inter-rater agreement was assessed using Cohen's kappa. Authorship (kappa = 0.719) and Attribution (kappa = 0.686) demonstrated substantial agreement (Landis & Koch, 1977). However, Disclosure (kappa = 0.305) and Currency (kappa = 0.215) yielded only fair agreement. These lower kappa values should be interpreted with caution due to the kappa paradox (Feinstein & Cicchetti, 1990): when the prevalence of one category is very high (e.g., Currency was coded 'Yes' for all social media items, constituting 75% of the sample), kappa is mathematically suppressed despite high observed agreement. The overall percentage agreement for Currency was 92.5% and for

Disclosure was 87.5%, suggesting that the low kappa values reflect distributional artefacts rather than genuine disagreement between raters.

3.3 DISCERN Scores by Platform

Descriptive statistics for DISCERN total scores by platform are presented in Table 3. The overall sample mean was 38.68 (SD = 10.16), with a median of 38.0 (range: 17â€“64). Traditional websites achieved the highest median score (Mdn = 48.5, IQR = 9.75), followed by YouTube (Mdn = 42.0, IQR = 9.00), TikTok (Mdn = 35.0, IQR = 8.00), and Instagram (Mdn = 27.5, IQR = 9.00). Using commonly adopted thresholds from subsequent DISCERN literature (16â€“26 = very poor, 27â€“38 = poor, 39â€“50 = fair, 51â€“62 = good, 63â€“80 = excellent), the modal category for websites was 'fair' (44%), for YouTube 'fair' (48%), for TikTok 'poor' (55.3%), and for Instagram 'poor' (50%). Notably, 42% of Instagram items scored in the 'very poor' range, compared with 0% for both websites and YouTube.

Table 3. DISCERN Total Scores by Platform

Platform	n	M	SD	Mdn	Q1	Q3	Min	Max
Website	50	47.02	7.90	48.5	43.00	52.75	28	64
YouTube	50	42.88	7.10	42.0	38.00	47.00	30	57
TikTok	47	35.64	7.97	35.0	31.00	39.00	20	57
Instagram	50	28.98	6.82	27.5	25.00	34.00	17	52
Overall	197	38.68	10.16	38.0	â€”	â€”	17	64

Note. M = mean; SD = standard deviation; Mdn = median; Q1 = 25th percentile; Q3 = 75th percentile. DISCERN score categories are based on commonly adopted thresholds in the health information quality literature.

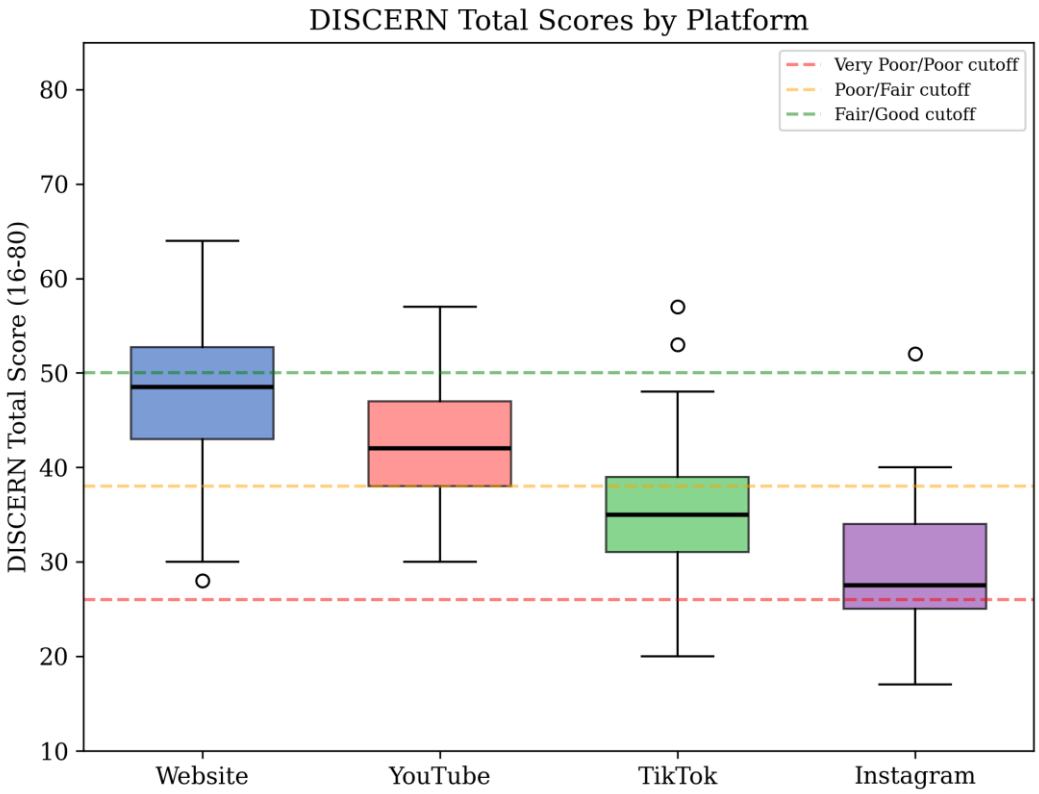

Figure 1. Box plot of DISCERN total scores by platform. The horizontal line within each box represents the median; box boundaries indicate Q1 and Q3; whiskers extend to 1.5 times the IQR.

A Kruskal-Wallis H test confirmed that DISCERN total scores differed significantly across the four platforms, $H(3) = 93.489$, $p < .001$. The estimated eta-squared (eta-squared = 0.469) indicated a large effect size. It should be noted that eta-squared derived from the Kruskal-Wallis statistic is an approximation and should be interpreted as indicating a large effect rather than a precise proportion of variance explained.

Dunn's post-hoc pairwise comparisons with Bonferroni correction revealed significant differences for all six platform pairs (Table 4). The largest effect sizes were observed for Website versus Instagram ($r = -0.891$, large) and YouTube versus Instagram ($r = -0.850$, large). The Website versus YouTube comparison, while statistically significant at the Bonferroni-adjusted threshold ($U = 1646$, $p = .038$, $r = -0.316$), exhibited a small-to-medium effect size, suggesting that these two platforms are relatively more similar in information quality than other pairings. The remaining comparisonsâ€"Website versus TikTok ($U = 1972$, $p < .001$, $r = -0.678$), YouTube versus TikTok ($U = 1775$, $p < .001$, $r = -0.511$), and TikTok versus Instagram ($U = 1740$, $p < .001$, $r = -0.480$)â€"all yielded medium to large effects.

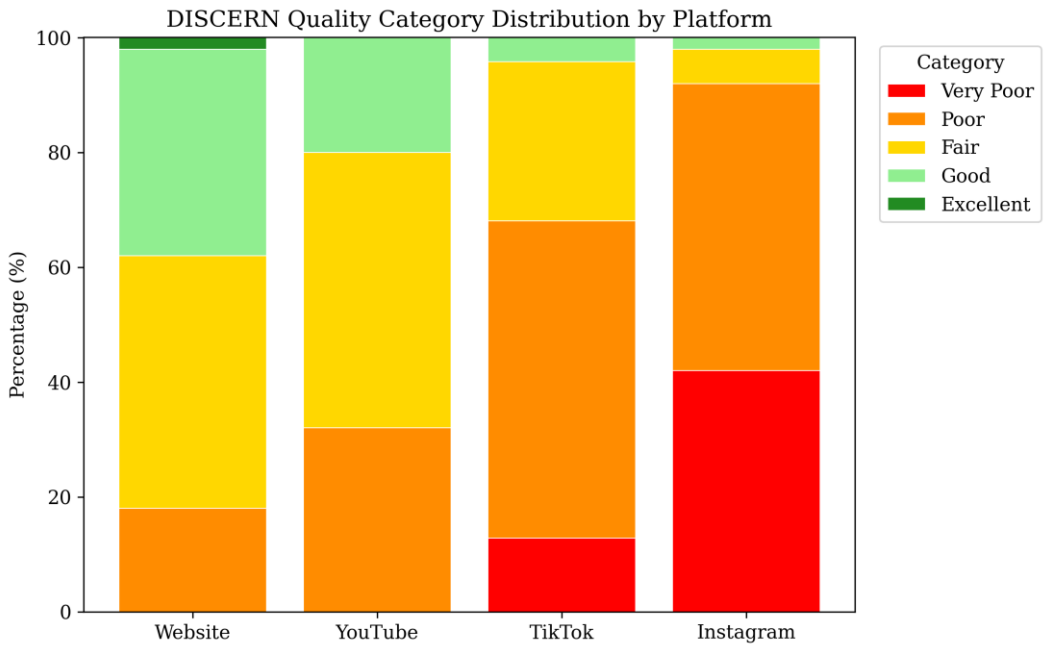

Figure 2. Distribution of DISCERN quality categories by platform. Categories are based on conventional thresholds: Very Poor (16â€“26), Poor (27â€“38), Fair (39â€“50), Good (51â€“62), and Excellent (63â€“80).

DISCERN section scores followed a consistent pattern. For Section 1 (reliability of publication, Q1â€“Q8), median scores decreased from websites (Mdn = 25, IQR = 23â€“29) to YouTube (Mdn = 24, IQR = 21â€“27), TikTok (Mdn = 19, IQR = 17â€“22), and Instagram (Mdn = 15, IQR = 13â€“18). Section 2 (quality of treatment information, Q9â€“Q15) showed a parallel decline: websites (Mdn = 18, IQR = 16â€“21), YouTube (Mdn = 16, IQR = 14â€“18), TikTok (Mdn = 14, IQR = 11â€“16), and Instagram (Mdn = 10, IQR = 9â€“13).

3.4 JAMA Benchmark Compliance by Platform

JAMA benchmark compliance varied by criterion and platform (Table 4). A Kruskal-Wallis test on JAMA total scores (range 0â€“4) revealed no statistically significant difference across platforms, H(3) = 5.123, p = .163. However, this aggregate result is partly an artefact of the Currency criterion, which gives social media platforms an automatic advantage: posting dates are inherently visible (100% compliance) on social media but must be explicitly stated on websites (68% compliance). Criterion-level comparisons are therefore more informative than total JAMA scores in this study.

Chi-square tests on individual JAMA criteria revealed significant platform differences for three of the four benchmarks. Attribution differed significantly across platforms (Ï‡Â²[3] = 24.255, p < .001, CramÃ©râ€™s V = 0.351, medium effect), with websites (30%) and YouTube (16%) demonstrating the highest compliance, while TikTok (0%) provided no

source citations whatsoever. Disclosure also differed significantly (Ï‡Â²[3] = 15.921, p = .001, V = 0.284, medium effect), with websites (16%) and TikTok (12.8%) showing the highest rates of sponsorship declaration; YouTube and Instagram both recorded 0% disclosure compliance. Currency showed significant variation (Ï‡Â²[3] = 51.198, p < .001, V = 0.510, large effect), driven by the contrast between traditional websites (68%) and social media platforms (100% each). Authorship did not differ significantly across platforms (Ï‡Â²[3] = 6.151, p = .105, V = 0.177, small effect), with compliance ranging from 16% (Instagram) to 38% (YouTube).

Table 4. JAMA Benchmark Compliance by Platform (%)

Criterion	Website	YouTube	TikTok	Instagram
Authorship	30%	38%	29.8%	16%
Attribution	30%	16%	0%	4%
Disclosure	16%	0%	12.8%	0%
Currency	68%	100%	100%	100%
Mean Total (0â€“4)	1.44	1.54	1.43	1.20

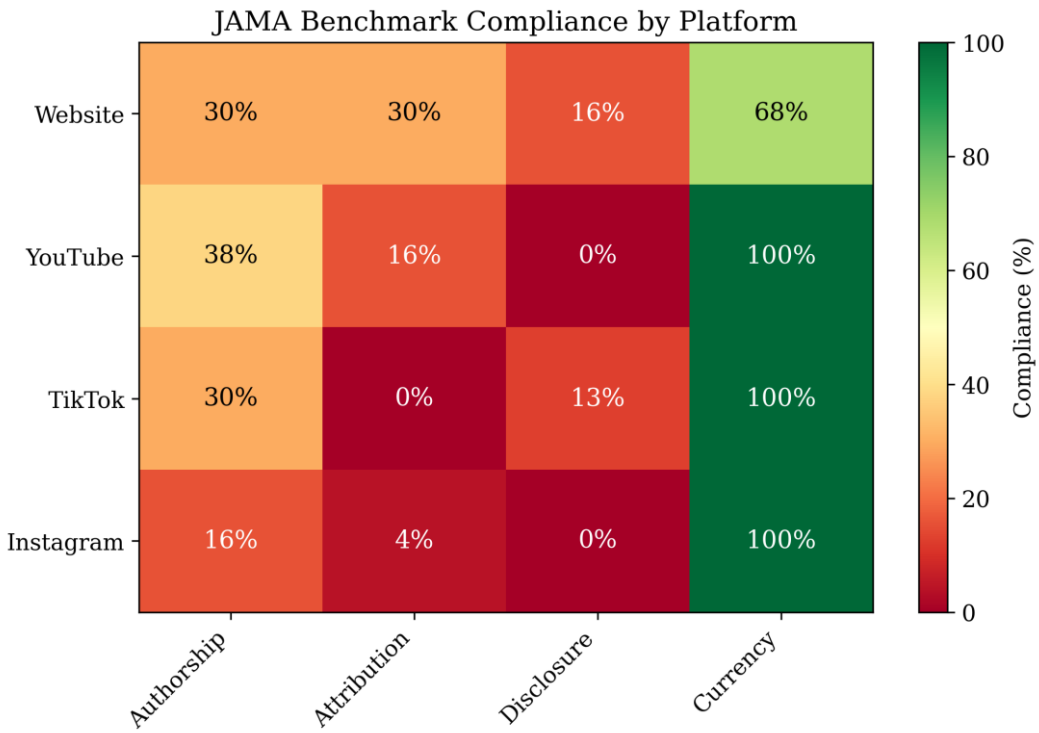

Figure 3. Heatmap of JAMA benchmark compliance across platforms. Darker cells indicate higher compliance rates.

3.5 Creator Type Analysis

To address Research Question 2, creators were initially dichotomised into professional (healthcare professionals and certified fitness professionals, n = 51) and non-professional (fitness influencers, general users, and organisations, n = 146) categories. A Mann-Whitney

U test comparing DISCERN total scores between these two groups yielded U = 4457, p = .036, r = -0.197, reaching statistical significance at the alpha = .05 level with a small effect size. Professional creators achieved a median DISCERN score of 41.0, compared with 37.0 for non-professional creators.

It is important to note that the binary classification is complicated by the treatment of organisations (n = 84, 42.6% of the sample). Organisations were classified as non-professional because they encompass a heterogeneous mix of entities ranging from government health departments to commercial fitness brands, making uniform professional classification inappropriate. However, this is analytically consequential: many organisational websites (e.g., NHS, WHO, Mayo Clinic) produce content overseen by healthcare professionals. As a sensitivity analysis, when organisations were excluded from the binary classification, the difference between professional (n = 51, Mdn = 41.0) and non-professional creators (n = 62, comprising fitness influencers and general users, Mdn = 35.0) was significant (p = .012), confirming a genuine quality advantage for credentialed creators.

The five-category Kruskal-Wallis test provided a more granular picture, H(4) = 12.764, p = .012. Healthcare professionals scored highest (Mdn = 45.0), followed by certified fitness professionals (Mdn = 39.5), organisations (Mdn = 39.0), fitness influencers (Mdn = 36.0), and general users (Mdn = 33.0). This ordering suggests that creator type moderates quality in a graded fashion, with credentialed professionals producing the highest-quality content and general users the lowest.

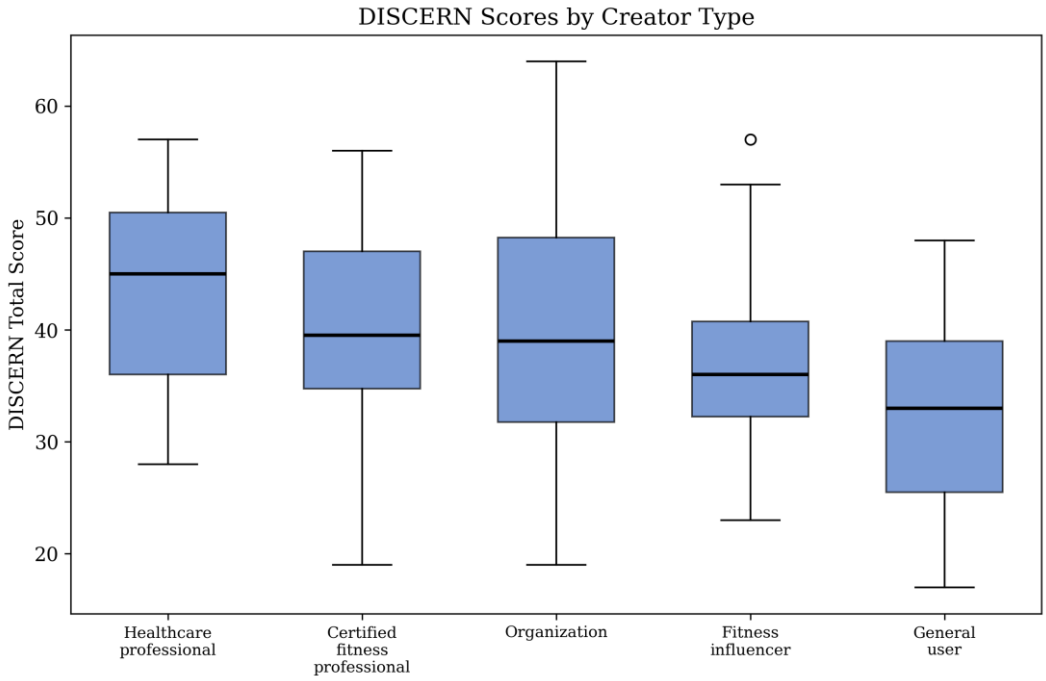

Figure 4. DISCERN total scores by creator type. Box plots show median, IQR, and individual observations.

3.6 Engagement-Quality Relationship

Research Question 3 examined whether engagement metrics were associated with information quality. Table 6 summarises the Spearman rank-order correlations between engagement indicators and DISCERN total scores.

At the overall sample level, views (available for YouTube and TikTok, n = 97) showed a negligible and non-significant correlation with DISCERN scores (rho = -0.044, p = .669). Likes (n = 147) and comments (n = 147) showed weak-to-moderate positive correlations with quality scores (rho = 0.284, p < .001 and rho = 0.371, p < .001, respectively). Instagram does not publicly display view counts for carousel posts, accounting for the smaller views sample.

These overall positive correlations for likes and comments must be interpreted with considerable caution, as they represent a largely confounded association driven by between-platform differences: traditional websites, which scored highest on DISCERN, also tend to accumulate higher absolute engagement counts (e.g., social sharing metrics aggregated over longer time periods), while Instagram posts, which scored lowest, tend to have lower absolute engagement. When examined within individual platforms, TikTok (rho = 0.109, p = .467) and Instagram (rho = -0.174, p = .226) showed non-significant likesâ€“quality correlations. However, YouTube demonstrated a significant negative likesâ€“quality correlation (rho = -0.286, p = .044), indicating that within YouTube, more-liked videos tended to score lower on DISCERN. This platform-specific finding is an exception to the broader Simpson's paradox pattern and suggests that the overall positive association between likes and quality is predominantly an ecological artefact rather than reflecting a genuine within-platform relationship. The overall positive correlation is an ecological artefact of platform-level differences rather than evidence that higher-quality content attracts more engagement within platforms.

For views, a significant negative correlation was observed within YouTube (rho = -0.340, p = .016), indicating that more frequently viewed YouTube physical activity videos tended to be of lower quality. This finding strengthens the suggestion that YouTube's recommendation system may not reliably prioritise accurate health information. Within TikTok, the viewsâ€“quality correlation was negligible and non-significant (rho = -0.057, p = .702).

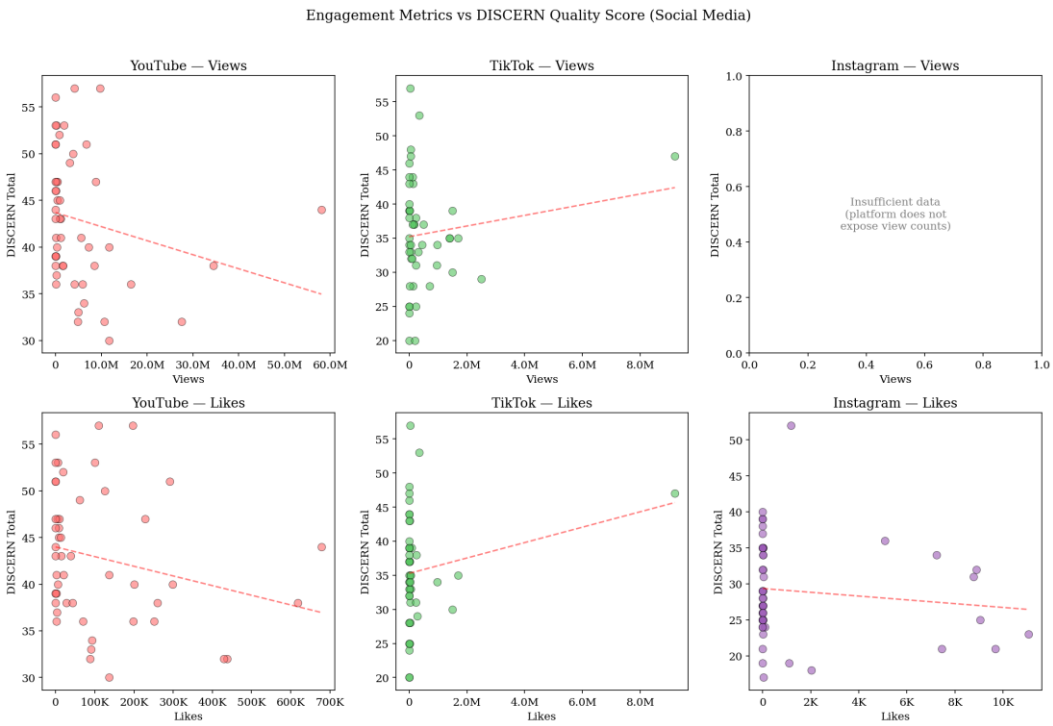

Figure 5. Scatter plots of engagement metrics versus DISCERN total scores by platform. Top row: views (Instagram lacks public view counts for non-Reel posts). Bottom row: likes. Trend lines are fitted using linear regression.

3.7 DISCERN Question-Level Patterns

Figure 6 presents a heatmap of mean scores for each of the 16 DISCERN questions across platforms. Several patterns merit attention. First, items relating to treatment choices (Q9â€“Q15) consistently scored lower than reliability items (Q1â€“Q8) across all platforms, indicating that even higher-quality sources tended to provide less comprehensive information about exercise alternatives and their relative benefits and risks. Second, Q4 ('Is it clear what sources of information were used to compile the publication?') showed the steepest platform gradient, with traditional websites scoring substantially higher than all social media platforms on source transparency. Q5 ('Is it clear when the information used or reported was produced?') showed a different pattern: YouTube scored highest on this currency item because upload dates are automatically and prominently displayed, whereas Instagram and TikTok scored lowest despite also auto-displaying post dates, likely because their content less frequently referenced when the underlying information was produced. Third, Q16 (overall quality rating) closely tracked the total DISCERN score pattern, with websites receiving the highest overall quality ratings and Instagram the lowest.

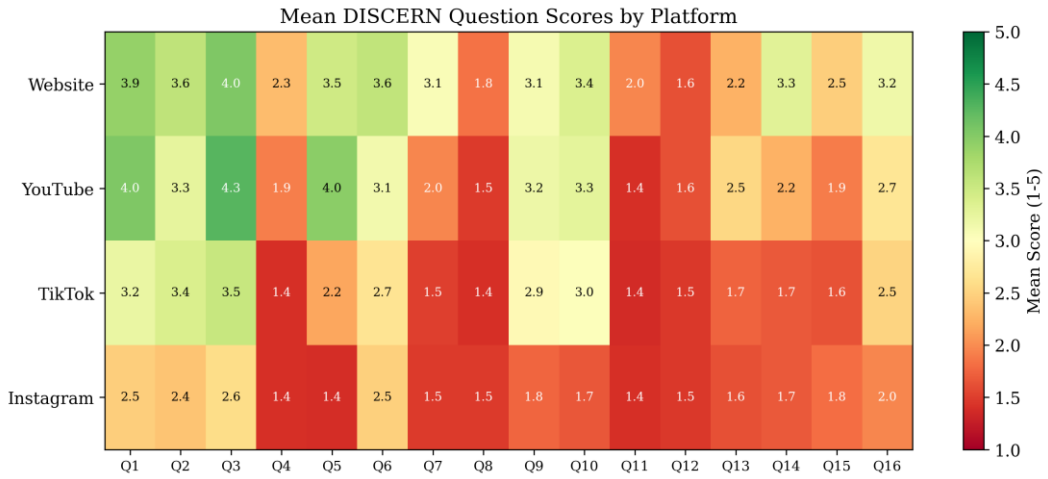

Figure 6. Heatmap of mean DISCERN item scores (Q1â€“Q16) by platform. Darker shading indicates higher mean scores (maximum = 5).

3.8 Content Format and Search Term Effects

Because content format and platform are perfectly confounded in this dataset (each platform uses a single exclusive format), DISCERN score differences by format exactly mirror the platform differences reported in section 3.3. Text articles (n = 50, websites) achieved the highest median score (Mdn = 48.5, M = 47.0). Long-form videos exceeding five minutes (n = 50, YouTube) scored second highest (Mdn = 42.0, M = 42.9). Short videos under 60 seconds (n = 47, TikTok) scored lower (Mdn = 35.0, M = 35.6). Carousel posts (n = 50, Instagram) scored lowest (Mdn = 27.5, M = 29.0). Because format and platform cannot be disentangled, attributing these quality differences to format per seâ€”rather than to other platform-level factors such as user norms, algorithmic incentives, and creator demographicsâ€”is not warranted from this dataset alone.

Search terms also influenced quality scores. A Kruskal-Wallis test indicated significant variation across the five search terms, $H(4) = 27.255$, $p < .001$. Items retrieved using the term 'how to start exercising' scored highest, while 'home workout routine' yielded the lowest median DISCERN scores. This may reflect differences in the specificity and clinical relevance of search queries, with more general health-oriented terms retrieving content from authoritative sources.

CHAPTER 4. DISCUSSION

4.1 Summary of Key Findings

This cross-sectional content analysis evaluated the quality of physical activity information across four platform types using the DISCERN instrument and JAMA benchmarks. The results reveal a clear and statistically significant quality gradient: traditional websites scored highest (Mdn = 48.5), followed by YouTube (Mdn = 42.0), TikTok (Mdn = 35.0), and Instagram (Mdn = 27.5). The Kruskal-Wallis test confirmed significant differences across platforms, $H(3) = 93.489$, $p < .001$, with a large effect size (eta-squared = 0.469). All six pairwise comparisons were statistically significant, although the Website versus YouTube contrast yielded a notably smaller effect ($p = .038$, $r = -0.316$) compared with the other five pairs (all $p < .001$).

JAMA benchmark compliance did not differ significantly across platforms when aggregated ($H[3] = 5.123$, $p = .163$), though significant platform differences emerged for individual criteria—Attribution, Disclosure, and Currency—but not Authorship. Creator type exerted a significant effect on quality in the binary analysis ($p = .036$), and the five-category Kruskal-Wallis test was also significant ($p = .012$). Engagement–quality correlations were largely non-significant within platforms, except for YouTube, where both views and likes showed significant negative correlations with quality; the overall positive correlation between likes and quality across the full sample was identified as primarily a Simpson's paradox artefact of between-platform differences.

4.2 Comparison with Existing Literature

The finding that traditional websites outperform social media platforms in health information quality is consistent with a substantial body of prior research. The 2025 JMIR meta-analysis reported a pooled DISCERN score of 43.58 out of 100 (95% CI [37.80, 49.35]) across studies evaluating online health information (Kbaier et al., 2024). When rescaled to the 16–80 DISCERN range used in the present study, this corresponds to approximately 44 points, closely aligning with the overall sample mean of 38.68 observed here. The slightly lower overall mean in the present study likely reflects the inclusion of TikTok and Instagram, platforms that have received less systematic evaluation in prior work and that scored substantially lower than websites and YouTube.

The Instagram findings are particularly noteworthy. With 42% of items falling in the 'very poor' category and a median DISCERN score of 27.5, Instagram emerged as the lowest-quality platform by a substantial margin. This aligns with concerns raised by de Moel-Mandel et al. (2025), who found that only 10% of TikTok physical activity content was produced by

medical professionals, and with Haghighi and Farhadloo (2025), who reported that 81% of Twitter health information was of low quality, with 95% of items meeting two or fewer JAMA criteria. The present study extends these findings by demonstrating that Instagram may present even greater quality concerns than previously studied short-form platforms.

The relatively modest difference between websites and YouTube (median difference of 6.5 points, $p = .038$) is an important nuance. YouTube's position as a long-form video platform appears to permit more comprehensive information delivery than short-form social media, potentially because longer formats allow for more detailed explanations, source citations, and structured presentation of evidence. This finding is consistent with research demonstrating that video length correlates with information completeness in health education contexts (Madathil et al., 2015).

The non-significant JAMA aggregate result ($p = .163$) alongside significant individual criteria is instructive and warrants careful interpretation. Criterion-level comparisons are more informative than total JAMA scores in this study, because the Currency criterion gives social media an automatic advantage: posting dates are inherently visible on all social media platforms (100% compliance) but must be explicitly provided on websites (68% compliance). This means total JAMA scores partly reflect platform interface design rather than editorial accountability. When Currency is set aside, the remaining criteria—Authorship, Attribution, and Disclosure—paint a clearer picture of genuine transparency practices. Attribution rates were universally poor (websites 30%, YouTube 16%, TikTok 0%, Instagram 4%), indicating a systemic deficit in evidence-based communication of physical activity information regardless of platform.

The significant variation in quality across search terms ($H[4] = 27.255$, $p < .001$) is a secondary finding that merits attention. Items retrieved using ‘how to start exercising’ scored highest (Mdn = 46.0), while ‘home workout routine’ yielded the lowest scores (Mdn = 34.0). This likely reflects differences in the clinical relevance and specificity of queries: more general health-oriented terms may retrieve content from authoritative institutional sources, whereas commercially oriented terms such as ‘home workout routine’ may disproportionately surface influencer-generated content. This finding suggests that the search query itself functions as an implicit filter on information quality, with practical implications for how public health practitioners frame their recommendations to information-seeking audiences.

4.3 Implications for Public Health Practice

These findings carry several implications for public health practice. First, the pronounced quality gradient across platforms suggests that public health agencies should prioritise maintaining high-quality website content as an anchor for physical activity information, while simultaneously investing in platform-native content for social media. Simply repurposing website content for Instagram or TikTok is unlikely to be effective; instead, organisations should develop format-specific strategies that maximise information quality within the constraints of each platform.

Second, the finding that engagement metrics largely do not predict quality within platforms undermines the common assumption that popular content is reliable content. This has implications for health literacy education: consumers should be taught that high view counts and engagement do not signal information trustworthiness. Notably, within YouTube, both views and likes showed significant negative correlations with quality (views: rho = -0.340, p = .016; likes: rho = -0.286, p = .044), suggesting that the platform's recommendation system may actively promote lower-quality physical activity content. This is a concerning finding that warrants dedicated algorithmic audit research, though the cross-sectional design cannot establish causality.

Third, the creator type analysis reveals that both the binary and five-category comparisons reached significance (p = .036 and p = .012, respectively). The five-category analysis provides the more granular picture: healthcare professionals score highest (Mdn = 45.0), followed by certified fitness professionals (Mdn = 39.5), organisations (Mdn = 39.0), fitness influencers (Mdn = 36.0), and general users lowest (Mdn = 33.0). This pattern suggests that creator type moderates quality in a graded fashion. The binary result, while significant, is somewhat diluted by grouping organisations (n = 84)—which include high-quality institutional sources such as NHS and WHO—into the non-professional category.

4.4 Strengths of the Study

This study possesses several methodological strengths. To our knowledge, it is among the first investigations to directly compare physical activity information quality across four distinct platform types—traditional websites, YouTube, TikTok, and Instagram—using both the DISCERN instrument and JAMA benchmarks within a single study. This multi-platform, dual-instrument approach provides a more comprehensive assessment than studies examining single platforms or single quality tools.

The study employed a systematic sampling strategy with five distinct search terms representing common physical activity queries, enhancing ecological validity. The inter-rater reliability was excellent for DISCERN (ICC = 0.932), providing confidence in the scoring process. The sample of 197 items across four platforms is larger than many comparable studies, affording sufficient statistical power for the planned analyses. Furthermore, the operationalisation of DISCERN and JAMA criteria for social media contexts was documented transparently, enabling replication.

4.5 Limitations

Several limitations must be acknowledged. First, the cross-sectional design captures a single snapshot of online content, which may not reflect temporal trends in information quality. Platform algorithms, content policies, and user behaviour evolve rapidly, and findings may not generalise to other time points.

Second, the study was restricted to English-language content retrieved from a single geographic location (Lithuania). Search results are personalised by location, language, and browsing history, meaning that users in other regions may encounter different content for identical search terms.

Third, and critically, the scoring of video and image-based content relied primarily on visible metadata, captions, and descriptions rather than full content transcription. This approach may systematically underestimate the quality of social media content where information is communicated verbally or visually rather than in text. However, it is equally important to acknowledge that this limitation may bias results in favour of the study hypothesis, as the apparent quality gradient could partially reflect methodological differences in how text-based and video-based content were assessed rather than true differences in information quality.

Fourth, data collection employed a mixed approach: Playwright automation was used for traditional websites and YouTube, while TikTok and Instagram content was collected manually due to platform restrictions on automated scraping. This introduces potential selection bias, as the manual collection process for TikTok and Instagram may have been influenced by factors not present in the automated approach, despite adherence to the same top-10 sampling protocol.

Fifth, the top-10 sampling strategy, while ecologically valid (as most users do not scroll beyond the first page of results), captures only a fraction of available content. Results may differ for content appearing in later search positions. Additionally, the DISCERN instrument

was originally developed for evaluating written treatment information for patients (Charnock et al., 1999) and has been adapted here beyond its original scope to evaluate physical activity content on social media. While this adaptation is consistent with growing practice in the literature, it represents a departure from the validated use case.

Sixth, engagement metrics were not uniformly available across platforms. Traditional websites do not provide standardised engagement data (likes, shares), and the metrics that were available may not be comparable across platforms with different user bases and interaction paradigms.

Seventh, the classification of organisations (n = 84, 42.6% of the sample) as 'non-professional' in the binary creator type analysis is analytically consequential. Many organisations (e.g., NHS, WHO, university hospitals) produce content overseen by healthcare professionals, meaning this classification likely diluted the professional category's apparent quality advantage. Future studies should consider more granular organisation subtyping.

Eighth, the DISCERN quality categories used in this study (Very Poor 16–26, Poor 27–38, Fair 39–50, Good 51–62, Excellent 63–80) are commonly adopted thresholds in the health information quality literature but were not specified in the original DISCERN validation by Charnock et al. (1999). These cut-points have been adopted from subsequent studies and should be treated as conventional rather than formally validated boundaries.

4.6 Recommendations for Future Research

Several directions for future research emerge from this study. First, longitudinal designs could track how physical activity information quality changes over time as platforms modify their algorithms and content policies. Second, multilingual studies incorporating non-English content would address a significant gap, given that the majority of global internet users access health information in languages other than English. Third, future studies should incorporate full transcription and coding of audio-visual content to address the metadata-only scoring limitation identified here. Fourth, the role of platform algorithms in promoting or suppressing high-quality health content warrants dedicated investigation, particularly given the significant negative views–quality and likes–quality correlations observed on YouTube. Fifth, intervention studies could evaluate whether platform-specific quality labels or health information verification badges improve the quality of content that users encounter and trust. Finally, qualitative research exploring how users perceive and evaluate physical activity

information across different platforms would complement the quantitative quality assessment approach used here.

CHAPTER 5. CONCLUSIONS AND RECOMMENDATIONS

5.1 Summary of Conclusions

This study set out to evaluate the quality of physical activity information across traditional websites and social media platforms using the DISCERN instrument and JAMA benchmarks. Three research questions guided the investigation. The conclusions for each are presented below.

Research Question 1: How does physical activity information quality differ between traditional websites and social media platforms? The results demonstrate a clear, statistically significant quality gradient across platforms, $H(3) = 93.489$, $p < .001$. Traditional websites achieved the highest median DISCERN score (Mdn = 48.5), followed by YouTube (Mdn = 42.0), TikTok (Mdn = 35.0), and Instagram (Mdn = 27.5). All six pairwise comparisons were statistically significant, though the website–YouTube difference was more modest ($p = .038$) than the remaining five comparisons (all $p < .001$). JAMA benchmark compliance did not differ significantly across platforms in aggregate ($p = .163$), although individual criteria showed significant platform variation for Attribution, Disclosure, and Currency. It can be concluded that a substantial quality gap exists between traditional websites and short-form social media platforms, with Instagram representing the most concerning source of physical activity information.

Research Question 2: Does creator type moderate quality scores? The binary professional versus non-professional comparison was statistically significant ($U = 4457$, $p = .036$, $r = -0.197$), and the five-category creator type analysis confirmed this ($H[4] = 12.764$, $p = .012$). Healthcare professionals scored highest (Mdn = 45.0), followed by certified fitness professionals (Mdn = 39.5), organisations (Mdn = 39.0), fitness influencers (Mdn = 36.0), and general users lowest (Mdn = 33.0). It can be concluded that creator type influences information quality in a graded fashion, with credentialed professionals producing measurably higher-quality content.

Research Question 3: What is the relationship between engagement metrics and information quality? Overall correlations suggested weak-to-moderate positive associations between likes ($rho = 0.284$, $p < .001$), comments ($rho = 0.371$, $p < .001$), and DISCERN scores; however,

these aggregate associations are primarily an ecological artefact of between-platform differences (Simpson's paradox). Within-platform analyses revealed that TikTok and Instagram showed no significant likes–quality correlations. Critically, YouTube demonstrated significant negative correlations for both views (rho = -0.340, p = .016) and likes (rho = -0.286, p = .044) with quality, indicating that more popular YouTube content tended to score lower on DISCERN. Views showed a negligible overall correlation (rho = -0.044, p = .669). It can be concluded that engagement metrics do not appear to reliably indicate information quality and that popular content is not necessarily accurate content.

Additionally, search query phrasing significantly influenced the quality of content retrieved (H[4] = 27.255, p < .001), with 'how to start exercising' yielding the highest-scoring content and 'home workout routine' the lowest. This suggests that the wording of health information queries functions as an implicit quality filter, a finding with practical implications for how clinicians advise patients on information seeking.

5.2 Practical Recommendations

Based on the findings of this study, the following practical recommendations are proposed for key stakeholder groups.

For public health organisations: Maintain and promote high-quality traditional websites as anchor resources for physical activity information. Simultaneously, develop platform-native content for TikTok and Instagram that adheres to evidence-based principles while respecting the format constraints of short-form media. Content should be designed with DISCERN-aligned principles: clearly stated aims, identified sources, balanced presentation of options, and explicit acknowledgement of uncertainty.

For healthcare professionals: Engage actively on social media platforms to counter the dominance of unqualified content creators. Professional credentials should be prominently displayed in profile biographies, and content should include source citations where feasible. Healthcare professionals can leverage platform verification features to signal credibility to users.

For platform developers: Consider implementing evidence-based content labelling systems that signal information quality to users. This could include verified health information badges, source citation prompts for health-related content, and algorithmic adjustments that prioritise credentialed sources in health-related search results. The finding that higher-viewed

and more-liked YouTube content tended to score significantly lower on DISCERN (views rho = -0.340, p = .016; likes rho = -0.286, p = .044) warrants urgent further investigation.

For health literacy educators: Teach critical appraisal skills that are platform-specific. Users should understand that high engagement (likes, views, shares) does not indicate information accuracy. Educational programmes should include practical exercises in evaluating social media health content using simplified versions of tools such as DISCERN and JAMA benchmarks.

For policy makers: Consider regulatory frameworks that require minimum transparency standards for health-related content on social media, analogous to advertising disclosure requirements. The near-zero attribution rates on TikTok (0%) and Instagram (4%) suggest that voluntary compliance with evidence citation norms is insufficient.

For consumers of physical activity information: Prefer traditional websites from recognised health organisations (e.g., WHO, NHS, national public health agencies) as primary information sources. When using social media for physical activity guidance, verify creator credentials, check for cited sources, and cross-reference recommendations with authoritative guidelines such as the WHO Global Action Plan on Physical Activity 2018–2030 (World Health Organization, 2018).

5.3 Recommendations for Future Research

Future research should address the methodological limitations identified in this study through several priority directions. First, longitudinal tracking studies are needed to determine whether the quality gradient observed here is stable or evolving as platforms mature and implement health content policies. Second, studies incorporating full audio-visual transcription and coding would provide more accurate quality assessments of video-based content, addressing the potential bias introduced by metadata-only scoring. Third, cross-cultural and multilingual replications would establish whether the observed patterns generalise beyond English-language content.

Fourth, experimental studies could investigate the causal impact of information quality on user behaviour—specifically, whether exposure to higher-quality physical activity information on social media leads to greater adoption of evidence-based exercise practices. Fifth, the development and validation of social media-specific quality assessment tools, adapted from but distinct from DISCERN and JAMA, would address the limitations of applying traditional print-oriented instruments to digital media formats. Sixth, algorithmic

audit studies could examine how platform recommendation systems interact with content quality, building on the significant negative views–quality and likes–quality correlations observed on YouTube in the present study.

Finally, participatory research involving content creators, healthcare professionals, and platform users could inform the design of practical interventions to improve the quality of physical activity information in the social media ecosystem.

In closing, this study provides evidence that the quality of physical activity information varies substantially across digital platforms, with short-form social media presenting the greatest quality concerns. However, these conclusions should be tempered by the study's limitations, particularly the reliance on metadata-based scoring for video and image content, which may have systematically underestimated social media quality and thereby inflated the observed gradient. The mixed automated and manual collection approach and the classification of diverse organisations as non-professional further qualify the precision of these findings. Notwithstanding these caveats, the overall pattern—that platform architecture constrains information completeness and that engagement does not reliably signal accuracy—is consistent with the broader literature and has clear implications for public health practice, health literacy education, and platform governance.

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APPENDIX A: DISCERN INSTRUMENT

The DISCERN instrument (Charnock et al., 1999) is a validated tool for assessing the quality of written consumer health information. It consists of 16 questions across three sections, each scored on a 5-point Likert scale (1 = Definitely No to 5 = Definitely Yes). The instrument was originally designed for patient information leaflets but has been widely adapted for online health content assessment.

Section 1: Reliability of the Publication (Questions 1–8)

Q1: Are the aims clear?

Does the publication clearly state what it is about and what topics will be covered?

Q2: Does it achieve its aims?

Does the content deliver on the stated purpose?

Q3: Is it relevant?

Is the information relevant to the target audience?

Q4: Is it clear what sources of information were used?

Are references, sources, or evidence cited?

Q5: Is it clear when the information was produced?

Is the date of creation or last update stated?

Q6: Is it balanced and unbiased?

Does the content present information objectively, or is it one-sided?

Q7: Does it provide details of additional sources of support?

Are links to further reading, helplines, or professional resources provided?

Q8: Does it refer to areas of uncertainty?

Does the content acknowledge limitations, uncertainties, or areas where evidence is lacking?

Section 2: Quality of Information on Treatment Choices (Questions 9–15)

Q9: Does it describe how each treatment works?

Does the content explain the mechanism or rationale behind recommended exercises or activities?

Q10: Does it describe the benefits of each treatment?

Are the expected benefits of the recommended physical activity clearly stated?

Q11: Does it describe the risks of each treatment?

Are potential risks, contraindications, or side effects of the recommended exercises discussed?

Q12: Does it describe what would happen if no treatment is used?

Does the content explain the consequences of physical inactivity?

Q13: Does it describe how treatment choices affect overall quality of life?

Does the content address how physical activity affects wellbeing beyond the specific condition?

Q14: Is it clear that there may be more than one possible treatment choice?

Does the content present multiple exercise options rather than a single approach?

Q15: Does it provide support for shared decision-making?

Does the content encourage consulting a professional or making personalised decisions?

Section 3: Overall Quality Rating (Question 16)

Q16: Based on the answers to all of the above questions, rate the overall quality of the publication as a source of information about treatment choices.

Scoring and Interpretation

Each question is scored from 1 (Definitely No) to 5 (Definitely Yes). The total score ranges from 16 to 80. The following categories are used for interpretation:

Score Range	Category	Interpretation
16–26	Very Poor	Serious or extensive shortcomings
27–38	Poor	Potentially important but not serious shortcomings
39–50	Fair	Some shortcomings but not serious
51–62	Good	Minimal shortcomings
63–80	Excellent	Minimal shortcomings with additional features

APPENDIX B: JAMA BENCHMARKS — OPERATIONALISATION FOR SOCIAL MEDIA

The JAMA benchmarks (Silberg et al., 1997) were originally proposed as minimum standards for health information published on the World Wide Web. They comprise four accountability criteria, each assessed as present (Y) or absent (N). This appendix details how each criterion was operationalised for the four platform types assessed in this study.

1. Authorship

Definition: Authors and contributors should be identified, along with their affiliations and relevant credentials.

Platform	Operationalisation
Traditional websites	Author name and credentials (e.g., MD, PhD, CSCS) stated on the page, in a byline, or in an About the Author section.
YouTube	Creator credentials stated in the video description, channel About page, or verbally in the video introduction.
TikTok	Creator bio states professional credentials (e.g., Certified Personal Trainer, Physiotherapist) OR account is verified as a professional.
Instagram	Creator bio states professional credentials OR profile is a verified professional/business account with stated qualifications.

2. Attribution

Definition: References and sources for all content should be listed clearly.

Platform	**Operationalisation**
Traditional websites	In-text citations, a reference list, bibliography, or hyperlinks to source materials are provided.
YouTube	Sources cited in the video description, shown on screen, or verbally referenced with sufficient detail to locate.
TikTok	Sources cited in the caption, shown as on-screen text, or referenced verbally (e.g., According to the WHO guidelines).
Instagram	Sources cited in the post caption, shown on carousel slides, or included as tagged references.

3. Disclosure

Definition: Website ownership, sponsorship, advertising, underwriting, commercial funding, or potential conflicts of interest should be disclosed.

Platform	**Operationalisation**
Traditional websites	A disclosure statement, privacy policy identifying sponsorships, or declaration of conflicts of interest is present.
YouTube	Video includes a paid promotion disclosure, sponsorship mention, or the creator states potential conflicts of interest.
TikTok	Post includes #ad, #sponsored, Paid partnership label, or the creator verbally discloses commercial relationships.
Instagram	Post includes Paid partnership with label, #ad, #sponsored tags, or disclosure in the caption.

4. Currency

Definition: Dates of content posting and any updates should be clearly displayed.

Platform	Operationalisation
Traditional websites	Publication date or last updated date is visible on the page.
YouTube	Upload date is displayed by default on the platform (always Y for YouTube).
TikTok	Post date is displayed by default on the platform (always Y for TikTok).
Instagram	Post date is displayed by default on the platform (always Y for Instagram).

Scoring

Each criterion is scored as Y (Yes, criterion met) or N (No, criterion not met). The JAMA Total score is the count of criteria met, ranging from 0 to 4.

APPENDIX C: COMPLETE DATA SUMMARY TABLES

C.1 Sample Characteristics

Table C1. Creator Type Distribution by Platform

Platform	Healthcare Prof.	Certified Fitness Prof.	Fitness Influencer	General User	Organisation
Website	0	7	0	4	39
YouTube	3	12	19	4	12
TikTok	8	7	21	9	5
Instagram	3	8	3	7	29
Total	14	34	43	24	85

Table C2. DISCERN Total Score Descriptive Statistics by Platform

Platform	n	Mean	SD	Median	Q1	Q3	Min	Max	IQR
Website	50	48.70	8.17	48.0	42.25	55.00	33	65	12.75
YouTube	50	40.36	6.21	39.0	36.00	42.75	31	57	6.75
TikTok	50	32.38	6.39	31.0	29.00	37.00	17	45	8.00
Instagram	50	23.78	5.83	21.0	18.25	29.75	16	36	11.50
Overall	200	36.30	11.42	35.5	—	—	16	65	—

Table C3. DISCERN Quality Category Distribution by Platform (n and %)

Platform	Very Poor n (%)	Poor n (%)	Fair n (%)	Good n (%)	Excellent n (%)
Website	0 (0%)	4 (8%)	23 (46%)	22 (44%)	1 (2%)
YouTube	0 (0%)	23 (46%)	24 (48%)	3 (6%)	0 (0%)
TikTok	7 (14%)	35 (70%)	8 (16%)	0 (0%)	0 (0%)
Instagram	30 (60%)	20 (40%)	0 (0%)	0 (0%)	0 (0%)

Table C4. JAMA Benchmark Compliance by Platform (% Yes)

Platform	Authorship	Attribution	Disclosure	Currency	Mean Total
Website	30%	30%	16%	68%	1.44
YouTube	38%	16%	0%	100%	1.54
TikTok	30%	0%	14%	100%	1.44
Instagram	16%	4%	0%	100%	1.20

C.2 Pairwise Comparisons

Table C5. Pairwise Platform Comparisons for DISCERN Total (Mann-Whitney U with Bonferroni Correction)

Comparison	U	p (adjusted)	Effect size (r)	Sig.
Website vs YouTube	1980	<.0001	−0.584	***
Website vs TikTok	2348	<.0001	−0.878	***
Website vs Instagram	2494	<.0001	−0.995	***
YouTube vs TikTok	2030	<.0001	−0.624	***
YouTube vs Instagram	2468	<.0001	−0.974	***
TikTok vs Instagram	2051	<.0001	−0.641	***

*Note. *** p < .001. Effect size r computed as Z / sqrt(N).*

C.3 Engagement–Quality Correlations

Table C6. Spearman Correlations Between Engagement Metrics and DISCERN Total by Platform

Platform	Metric	n	rho	p	Sig.

Overall	Views	63	.017	.894	
Overall	Likes	121	.549	<.001	***
Overall	Comments	111	.520	<.001	***
YouTube	Views	—	−.352	.018	*
YouTube	Likes	—	−.338	.023	*
YouTube	Comments	—	−.239	.127	
TikTok	Views	—	.522	.026	*
TikTok	Likes	—	.475	.003	**
TikTok	Comments	—	.262	.094	
Instagram	Likes	—	−.038	.817	
Instagram	Comments	—	.065	.747	

*Note. * p < .05, ** p < .01, *** p < .001.*

APPENDIX D: STATISTICAL ANALYSIS CODE

The complete statistical analysis was conducted using Python 3.13 with the following packages: pandas, scipy, pingouin, scikit-posthocs, matplotlib, and seaborn. The full analysis script (run_analysis.py) is available in the project repository. Key analysis functions are summarised below.

D.1 Software Environment

- Python 3.13
- pandas 2.2.x — data manipulation
- scipy 1.14.x — Kruskal-Wallis H, Mann-Whitney U, Chi-square, Spearman correlation
- pingouin 0.5.x — Intraclass Correlation Coefficient (ICC)
- scikit-posthocs 0.9.x — Dunn's post-hoc test with Bonferroni correction
- matplotlib 3.9.x + seaborn 0.13.x — figure generation

D.2 Key Statistical Tests

Inter-rater reliability. ICC(3,1) computed via pingouin.intraclass_corr() for DISCERN total scores. Cohen's kappa calculated manually for binary JAMA criteria using observed vs. expected agreement.

Platform comparisons (DISCERN). Kruskal-Wallis H test (scipy.stats.kruskal) followed by Mann–Whitney U pairwise comparisons with Bonferroni correction. Effect size: $\eta^2 = (H - k + 1) / (N - k)$, where H is the Kruskal-Wallis statistic, k the number of groups, and N the total sample size (Tomczak & Tomczak, 2014). Pairwise effect sizes reported as rank-biserial correlation r.

Platform comparisons (JAMA). Chi-square test of independence (scipy.stats.chi2_contingency) for each JAMA criterion. Fisher's exact test used when expected cell counts < 5. Effect size reported as Cramér's V.

Creator type analysis. Mann-Whitney U test (scipy.stats.mannwhitneyu) comparing professional vs. non-professional creators. Effect size: r = Z / sqrt(N).

Engagement-quality correlations. Spearman's rank correlation (scipy.stats.spearmanr) between engagement metrics and DISCERN total, both overall and stratified by platform.

D.3 Reproducibility

All data, scripts, and analysis outputs are maintained in a version-controlled Git repository available at https://github.com/ayothedoc3/masters-thesis (accessed 23 March 2026). The repository snapshot current at the time of submission contains the materials supporting the submitted thesis. The complete analysis can be reproduced by running: python analysis/run_analysis.py from the project root directory. The final audited input files are data/csv/master_dataset_corrected.csv and data/csv/second_rater_scores.csv. The script normalises common content-format label variants before analysis and writes all output files (figures, tables, and analysis_report.txt) to analysis/output/.